NRSG 233 HEALTH ASSESSMENT · MODULE 3

Skin, Hair & Nails
Complete study guide

For Luke's Module 3 quiz on Tuesday, October 13, 2026, 8:30 am PT.

> **Personal study sheet, not for submission.** NRSG 233 bars AI on submitted work. The Braden practice and the Module 3 documentation sheet are not filled in here.

Built from: (1) her lecture slides, "Skin, Hair, and Nails" (Ysis Mercado, 46 slides) as the **primary source**, in slide order, cited [Slide N]; (2) her handouts: the Skin Lesion Reference Guide (Relias/WCEI) and the Braden Scale; (3) Jarvis & Eckhardt 9th ed., prioritizing her readings: pp. 111, 207–215, 226–237, 246 and Ch. 21 pp. 519–520, with Tables 13.1–13.5 and 13.10 as focus; (4) the Module 3 vocabulary list (31 terms).

Blue boxes = her slides. If her slides and Jarvis differ, hers come first.

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- Appendix B: Braden Scale handout (hers)

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0. Her Objectives

> [Slide 3] Objectives — by the end you should be able to:
>
> - Perform general inspection of the skin and use palpation to assess for temperature, turgor, moisture.
> - Describe the assessments of the scalp, hair, and nails.
> - Distinguish between normal and abnormal findings of the integumentary exam and use appropriate terminology to document findings.
> - Demonstrate appropriate assessment and evaluation for common hospital skin concerns including pressure and edema.
> - Describe the appropriate nursing actions for any abnormal findings.

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1. What She Emphasized

- **Objectives above are the exam frame** — inspect, palpate, document with correct terminology, handle pressure/edema, and know nursing actions for abnormal findings [Slide 3].
- **"Consciously attend to skin characteristics; the danger is one of omission"** — you miss what you don't systematically look at [Slide

5].

- **"OFTEN ARE PREVENTABLE!"** — pressure injuries; also **"ESSENTIAL TO PERFORM A THOROUGH ASSESSMENT!"** [Slide 40].
- **USE THESE TERMS TO DESCRIBE LESIONS** — configurations: annular/circular, confluent, discrete, grouped, linear, target, zosteriform (in caps on her slides) [Slides 18–21]; more terms in [Lesion handout].
- **ABCDEF, not ABCDE** — danger signs in pigmented lesions: A asymmetry, B border, C color, D diameter, E elevation or evolution, F funny looking [Slide 12].
- **Braden: six subscales** — Sensory Perception, Moisture, Activity, Mobility, Nutrition, Friction & Shear [Slide 41]; score bands: MILD 15–18, MODERATE 13–14, HIGH 10–12, SEVERE <9 [Slide 43].
- **Practice describing lesions** — repeated two slides ("How would you describe this?") with the same documentation list: color, elevation, pattern/shape, size, location/distribution, exudate (color and odor) [Slides 36–37; also 17].
- **Pitting edema grades** — 1+ mild, 2+ moderate, 3+ deep, 4+ very deep; know pitting vs non-pitting [Slide 15]; assessment photo/demonstration [Slide 16].
- **Where to assess turgor** — she asked it as a question, so expect it tested [Slide 14]; Jarvis site = anterior chest under the clavicle [Jarvis p. 210].
- **Module 3 Documentation Sheet items** — nail base angle options 160°, <160°, ≥180°; capillary refill <2 seconds vs >2 seconds [Module 3 documentation sheet].
- **Readings focus** — Tables 13.1, 13.2, 13.3, 13.4, 13.5, 13.10 and pages 111, 207–215, 226–237, 246, and Ch 21 pp. 518–519–520 [assigned pages / Module 3 sheet; text from Jarvis 2024, Slide 46].
- **Hands-on equipment** — gloves, penlight, small centimeter ruler [Slide 5]; a ruler for lesion size in cm is also what Jarvis requires [Jarvis p. 211].
- **Skin tears: "Handle with care!"** — loss of collagen with aging increases risk [Slide 38].

2. Skin Overview and Functions

[Slide 4] SKIN: OVERVIEW AND FUNCTIONS

- Largest organ in the body, covers 20 square feet.
- Protection; defense barrier against invasion of microorganisms.
- Fluid balance.
- Perception (listed twice on the slide).
- Wound repair.
- Temperature regulation.
- Absorption and excretion.
- Production of vitamin D.
- Communication.
- Identification.

Jarvis fill-in: the assigned excerpts do not include the skin anatomy figure, so learn layers through how deep lesions go: **epidermis** (superficial; lost in erosion) → **dermis** (ulcer extends into it and heals with a scar) → **subcutaneous layer** (cyst sits here; pressure injury stage 3 reaches subcutaneous tissue) [Jarvis pp. 230–232, 234]. Also know **stratum corneum** = compacted desquamated layers shed as scale [Lesion handout].

3. Preparation and Technique

[Slide 5] PREPARATION

- Consciously attend to skin characteristics; **the danger is one of omission**.
- Provide adequate lighting.
- Have client sit, wearing gown and drape — allows easy visibility of skin.
- Equipment: gloves, penlight, and small centimeter ruler.

Palpation — which part of the hand for what [Jarvis p. 111]:

- **Fingertips** — fine tactile discrimination: skin texture, swelling, pulsation, lumps.
- **Grasping action of fingers and thumb** — position, shape, consistency of an organ or mass.
- **Dorsa (backs) of hands and fingers** — temperature (skin thinner than palms).
- **Base of fingers (metacarpophalangeal joints) or ulnar surface of the hand** — vibration.
- Technique: slow, systematic, calm, gentle; warm your hands; palpate tender areas last; light palpation first, then deeper; intermittent pressure with deep palpation [Jarvis p. 111].

Examination approach [Jarvis p. 207]: help the person remove clothing and assess the skin as one entity; stand back first for an overall impression to reveal distribution patterns; then inspect lesions carefully; with a rash check **all** areas of the body, plus mucous membranes. Always remove socks and inspect feet, toenails, and folds between the toes.

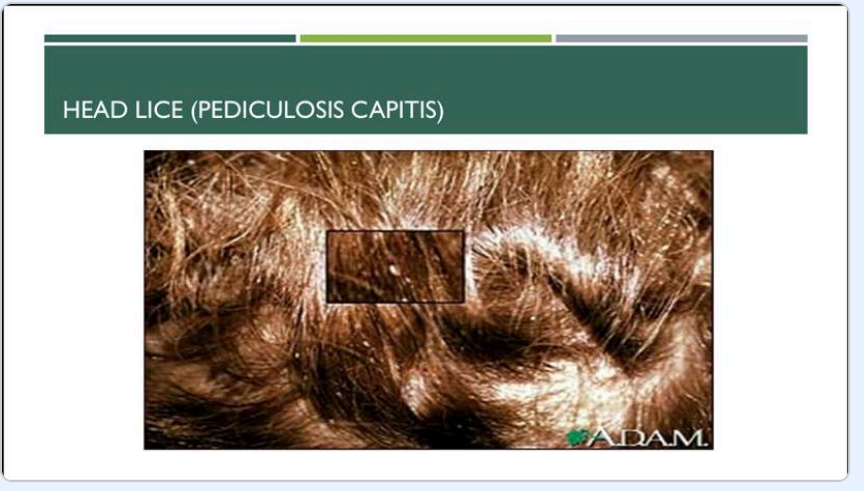
> Jarvis also stresses good lighting, adequate exposure, comparing right vs left sides as the person's own control, and that inspection always comes first [Jarvis p. 111] — matches her "adequate lighting" and gown/drape points [Slide 5].

4. Hair

[Slide 6] INSPECT & PALPATE THE HAIR

- Color
- Texture
- Distribution
- Lesions

[Slide 7] HEAD LICE (PEDICULOSIS CAPITIS) — title/photo slide; fill in from Jarvis.



Head lice (pediculosis capitis) [Slide 7]

Jarvis fill-in [Jarvis p. 213; Table 13.10, p. 246]:

TABLE 13.10 Abnormal Conditions of Hair



◀ Toxic Alopecia

Patchy, asymmetric balding that accompanies severe illness or use of chemotherapy in which growing hairs are lost and resting hairs are spared. Regrowth occurs after illness or discontinuation of toxic chemotherapy.



◀ Tinea Capitis (Scalp Ringworm)

Rounded, patchy hair loss on scalp, leaving broken-off hairs, pustules, and scales on skin. Caused by fungal infection; lesions fluoresce bright blue-green under Wood's light. Usually seen in children and farmers; highly contagious; may be transmitted by another person, by domestic animals, or from soil.



◀ Traction Alopecia

This hair loss affects one-third of women of African descent, and the cause is mechanical.[4] The "fringe sign" is marginal alopecia along the frontal or temporal hairline, where the hair is in constant tension, but loss can occur in any scalp area. Trauma is from tight braiding, tight ponytail, barrettes, cornrows, hair weaves, dreadlocks, and use of chemical relaxation. May see broken hairs or pustules in follicles. Black hair is intrinsically fragile, and the continuous pulling in one direction can break hair and cause loss. Improves with loosening the braids, redoing braids after 2 to 3 months in a different direction, or taking hairstyle out completely for 2 to 3 months.

Continued

Table 13.10 Abnormal conditions of hair [Jarvis p. 245]

TABLE 13.10 Abnormal Conditions of Hair—cont'd



Seborrheic Dermatitis (Cradle Cap)

Thick, yellow-to-white, greasy, adherent scales with mild erythema on scalp and forehead; very common in early infancy. Resembles eczema lesions, except that cradle cap is distinguished by absence of pruritus, presence of "greasy" yellow-pink lesions, and negative family history of allergy.



Folliculitis Barbae ("Razor Bumps")

Superficial inflammatory infection of hair follicles. Multiple pustules, "whiteheads," with hair visible at center and erythematous base. Usually involves face and neck and is common in Black and Latino men with tight curly hair. Occurs after shaving when growing out hairs curl in on themselves and pierce the skin, making a foreign-body inflammatory reaction.



Pediculosis Capitis (Head Lice)

Head lice infestation is common in preschoolers and school-aged children and has no correlation with socioeconomic status. The parasitic lice need close head-to-head contact for transmission (during play or sleep); they feed off the scalp and lay eggs along the hair shaft.[11] Nits (eggs) are easy to see on hairs close to the scalp; they are translucent, teardrop in shape, clear to light brown, and adherent to hair shafts. Check behind ears and nape of neck. The condition is intensely pruritic, so a child frequently scratching the scalp is a good clue. Many over-the-counter products (pediculicides) are available for treatment.

Continued

Table 13.10 cont'd [Jarvis p. 246]

- **Color** — melanin production, pale blond to total black; graying may begin as early as the 30s (reduced melanin in follicles).
- **Texture** — fine or thick; straight, curly, or kinky; normally shiny (may be lost with dyes, rinses, permanents). Abnormal: dull, coarse, or brittle hair. Gray, scaly, well-defined areas with broken hairs = tinea capitis (school-age children). Loss of eyebrows/scalp hair with chemotherapy or hypothyroidism.
- **Distribution** — vellus hair on body; terminal hair at eyebrows, lashes, scalp; adult male/female patterns (female inverted triangle; male upright triangle extending toward umbilicus). Abnormal: sparse genital hair (endocrine), hirsutism (androgen excess).

- **Lesions/scalp** — part hair in sections and lift; with itching check behind the ears and the occipital area. Seborrhea (dandruff) = loose white flakes and is common/normal.
- **Pediculosis capitis (head lice)** [Jarvis p. 246] — common in preschoolers and school-age children, **no correlation with socioeconomic status**; spread by close head-to-head contact (play or sleep); lice feed on the scalp and lay eggs on the hair shaft. **Nits** = translucent, teardrop-shaped, clear to light brown, adherent to the hair shaft, easiest to see near the scalp — **distinguish from dandruff**. Check behind the ears and nape of neck. Intensely pruritic (child frequently scratching is the clue). Treat with OTC pediculicides.

5. Nails

TABLE 13.11 Abnormal Conditions of the Nails



Scabies

An intensely pruritic contagion caused by the scabies mite. Mites form a linear or curved elevated burrow on the fingers, web spaces of hands, and wrists. Highly contagious. Severe itching causes sleep disturbance and bacterial skin infections. A common communicable disease in resource-poor countries. Dermatoscope viewing shows exactly where the mite is in the burrow, which eases removal with a scalpel.



Paronychia

Red, swollen, tender inflammation of the nail folds. Acute paronychia is usually a bacterial infection with pus in the proximal nail fold, pain, and throbbing. Chronic paronychia is most often a fungal infection from a break in the cuticle in those who perform "wet" work.



Beau Lines

Transverse furrow or groove. A depression across the nail that extends down to the nail bed. Occurs with any trauma that temporarily impairs nail formation such as acute illness, toxic reaction, or local trauma. Dent appears first at cuticle and moves forward as nail grows.



Splinter Hemorrhages

Red-brown linear streaks from damage to nail bed capillaries. They occur with systemic diseases (vasculitis), trauma or sports-related injuries, and endocarditis.

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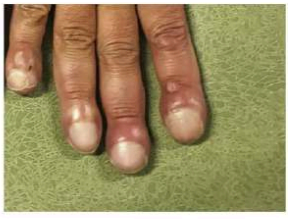
Table 13.11 Abnormal conditions of the nails [Jarvis p. 248]

TABLE 13.11 | Abnormal Conditions of the Nails—cont'd



Onychomycosis

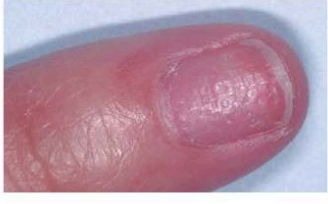
This is a slow, persistent fungal infection of fingernails and, more often, toenails, common in older adults. Fungus causes change in color (green where nail plate separated from bed), texture, and thickness, with nail crumbling or breaking and loosening of the nail plate, usually beginning at the distal edge and progressing proximally.



Late Clubbing

Inner edge of nail elevates; nail bed angle is greater than 180 degrees. Distal phalanx looks rounder, wider, and shiny.

Chronic lung inflammation, lung cancers, heart defects with right-to-left shunts may cause release of growth factors (e.g., platelet-derived growth factor) and promote growth of vessels. Clubbing usually develops slowly over years; if the primary disease is treated, clubbing can reverse.



Pitting

Sharply defined pitting and crumbling of nails with distal detachment often occurs with psoriasis.



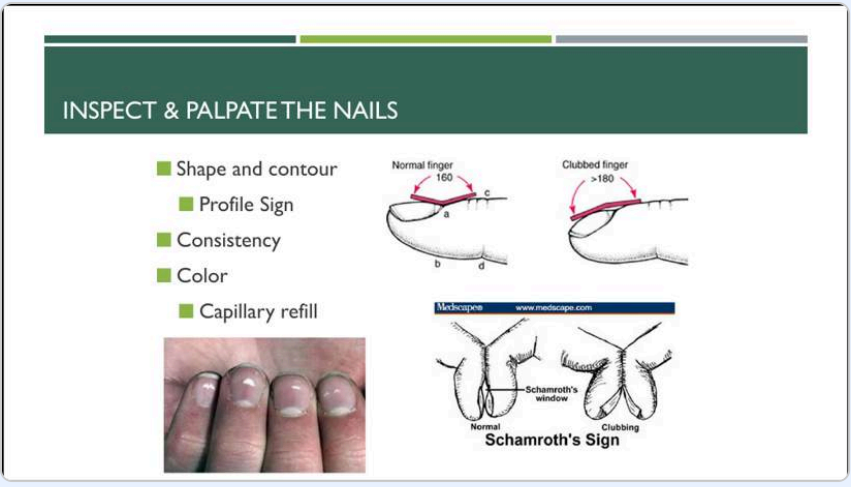
Habit-Tic Dystrophy

Depression down middle of nail or multiple horizontal ridges caused by continuous picking of cuticle by another finger of same hand, which causes injury to nail base and nail matrix.

See Illustration Credits for source information.

Table 13.11 cont'd [Jarvis p. 249]

[Slide 8] INSPECT & PALPATE THE NAILS



Nails: profile sign 160° vs clubbing >180°, Schamroth sign [Slide 8]

- Shape and contour — **Profile Sign**
- Consistency
- Color
- Capillary refill

Jarvis fill-in [Jarvis pp. 213–215]:

- **Shape/contour** — nail surface slightly curved or flat; posterior and lateral nail folds smooth and rounded; nail edges smooth, rounded, clean (suggests adequate self-care). Jagged/bitten nails or traumatized folds suggest nervous picking.
- **Profile sign** — view the index finger in profile; nail base angle should be about **160°**; nail base is firm to palpation. Curved (convex) nails are a normal variant if the angle is 160° or less.
- **Clubbing** — early clubbing: angle straightens to **180°** and nail base feels **spongy**; later the nail becomes convex. Associated with congenital cyanotic heart disease, lung cancer, pulmonary disease [Jarvis p. 214].
- **Consistency** — normally smooth, regular, not brittle or splitting; uniform thickness; nail firmly adheres to the nail bed. Abnormal: pits, transverse grooves/lines (nutrient deficiency or acute illness); thickened and ridged nails with arterial insufficiency.
- **Color** — translucent plate over an even, pink nail bed. Leukonychia (white hairline lines from trauma/picking) is normal in everyone; dark-skinned people may have brown-black bands/streaks.

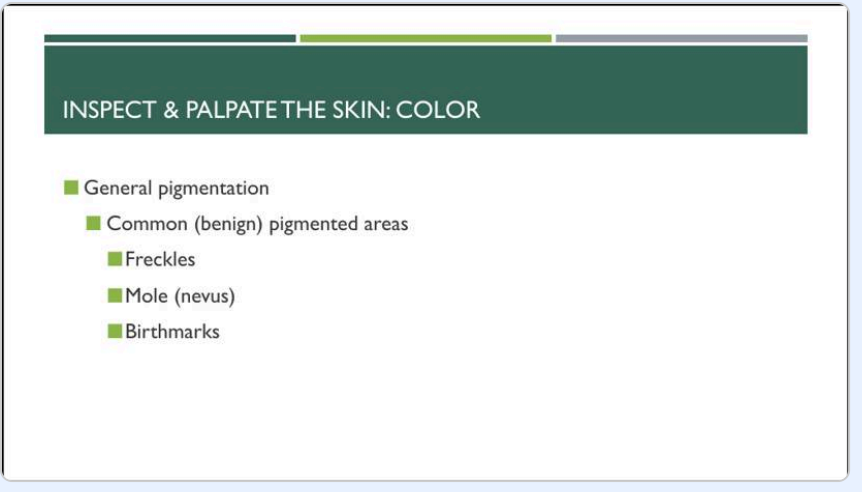
Abnormal: cyanosis or marked pallor, splinter hemorrhages, transverse ridges, Beau lines. Suspect melanoma with darkly pigmented nail bands, varied pigmentation, width >3 mm in light-skinned persons.

- **Capillary refill** — index or middle fingertip at heart level; depress nail edge at least 5 seconds to blanch, release, note color return; normally instant/within a few seconds (a healthy return takes **1 or 2 seconds**); >3 seconds indicates clinical decline (cardiovascular/respiratory failure, septic shock) [Jarvis p. 215].

> Her documentation sheet uses the categories **160° / <160° / ≥180°** and capillary refill **<2 seconds / >2 seconds** [Module 3 documentation sheet]. Jarvis frames the same findings as ~160° normal, 180° early clubbing, and 1–2 seconds healthy refill with >3 seconds abnormal [Jarvis pp. 214–215].

6. Skin Color

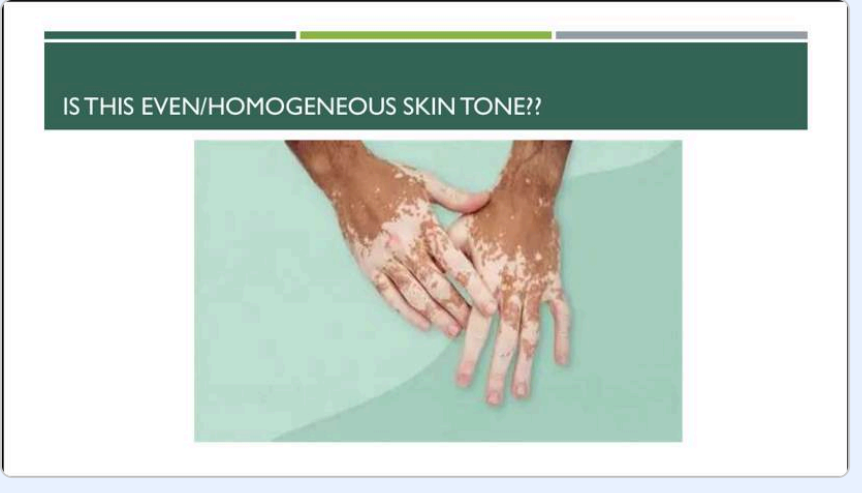
[Slide 9] INSPECT & PALPATE THE SKIN: COLOR



Common pigmented areas [Slide 9]

- General pigmentation
- Common (benign) pigmented areas: freckles; mole (nevus); birthmarks

[Slide 10] "IS THIS EVEN/HOMOGENEOUS SKIN TONE??" — photo/discussion slide; normal = even, homogeneous tone.



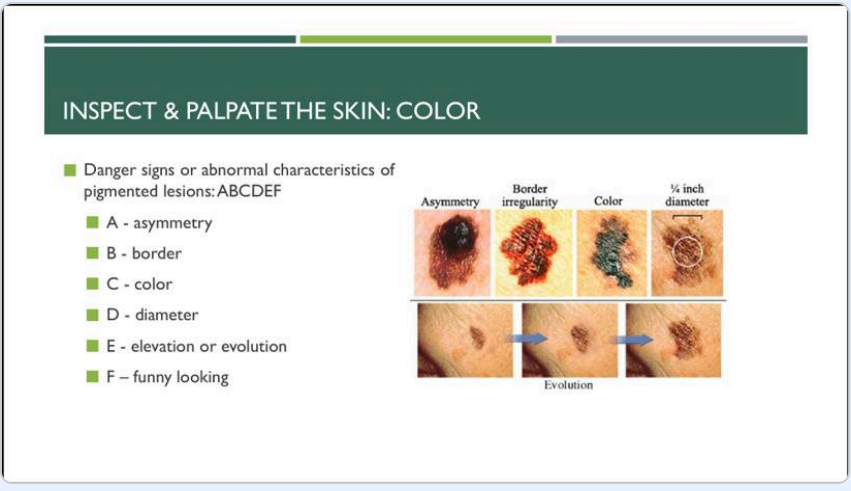
Is this even/homogeneous skin tone? [Slide 10]

[Slide 11] Widespread color change definitions:

- **Pallor** — loss of color; the red-pink tones from oxygenated hemoglobin in the blood are lost.
- **Erythema** — intense redness of skin; due to excess blood in dilated superficial capillaries.

- **Cyanosis** — bluish mottled color; signifies decreased perfusion of oxygenated blood to the tissues.
- **Jaundice** — yellowish skin color; indicates rising amounts of bilirubin in the blood.

[Slide 12] Danger signs / abnormal characteristics of pigmented lesions: **ABCDEF**



ABCDEF danger signs [Slide 12]

- A – asymmetry; B – border; C – color; D – diameter; E – elevation or evolution; F – funny looking.

Jarvis fill-in [Jarvis pp. 207–209]:

- **General pigmentation** — normally even and consistent with genetic background; pinkish tan to ruddy dark tan or light to dark brown, possibly yellow/olive overtones; darker in sun-exposed areas. Benign: **freckles** (small flat brown macules on sun-exposed skin); **moles/nevi** (clump of melanocytes, tan-brown, flat or raised; acquired nevi = symmetric, ≤6 mm, smooth borders, uniform color); **birthmarks** (tan to brown). Abnormal: **vitiligo** — patchy absence of melanin on face, neck, hands, feet, body folds, around orifices; more severe impact in dark-skinned people [Jarvis p. 207].
- **ABCDEF details** [Jarvis pp. 207–208] — **A**symmetry (two halves look different); **B**order irregularity (notching, scalloping, ragged, poorly defined); **C**olor variation (brown, tan, black, blue, red, white, or combination); **D**iameter >6 mm (pencil eraser), though early melanomas may be smaller; **E**levation or **E**volution (sudden appearance or recent change); **F**unny looking = "ugly duckling" sign, the lesion that looks different from its neighbors. Additional

warning symptoms: rapidly changing lesion, new pigmented lesion, itching/burning/bleeding in a mole — warrant referral.

Light vs dark skin (Table 13.1 [Jarvis pp. 226–228]):

Change	Light skin	Dark skin
Pallor	Generalized pallor	Yellow-brown or ashen gray, dull; check conjunctivae/mucous membranes
Cyanosis	Dusky blue	Dark, dull, lifeless; only severe cyanosis visible; check conjunctivae, oral mucosa, nail beds
Erythema	Red, bright pink	Purplish tinge, hard to see; **palpate** for warmth, taut skin, deep hardening
Jaundice	Yellow in sclera, hard palate, mucosa, then skin	Sclera yellow up to iris; best at hard/soft palate junction and palms

> Reliable sites for color change are those with least pigmentation: under the tongue, buccal mucosa, palpebral conjunctiva, sclera — lips and nail beds may be inaccurate in dark-skinned people [Jarvis p. 208]. Pallor in dark skin = absence of the luster of the underlying red tones; jaundice is best assessed in direct natural daylight; calluses on palms/soles look yellow but are not jaundice; do not confuse scleral fatty deposits with jaundice [Jarvis pp. 208–209].

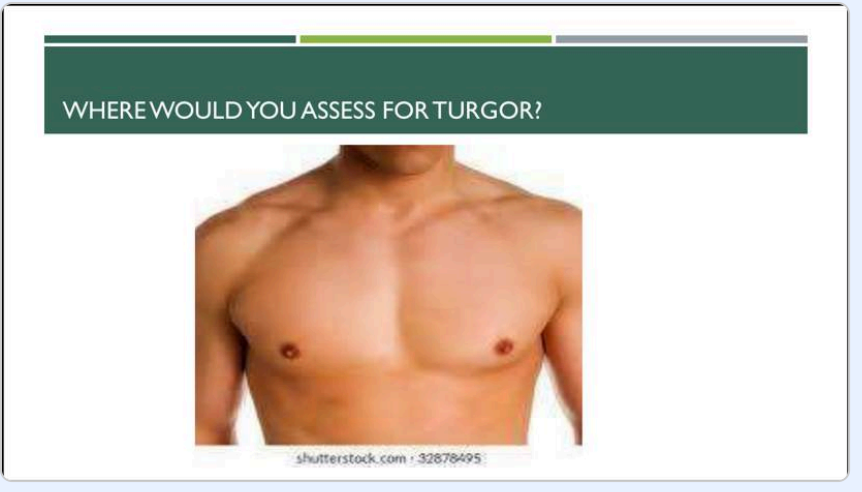
7. Temperature, Moisture, Texture, Thickness,

Edema, Turgor, Vascularity

[Slide 13] INSPECT & PALPATE THE SKIN

- Temperature; Moisture; Texture; Thickness; Edema; Mobility and Turgor; Vascularity or Bruising; Lesions.

[Slide 14] "WHERE WOULD YOU ASSESS FOR TURGOR?" — know the site.

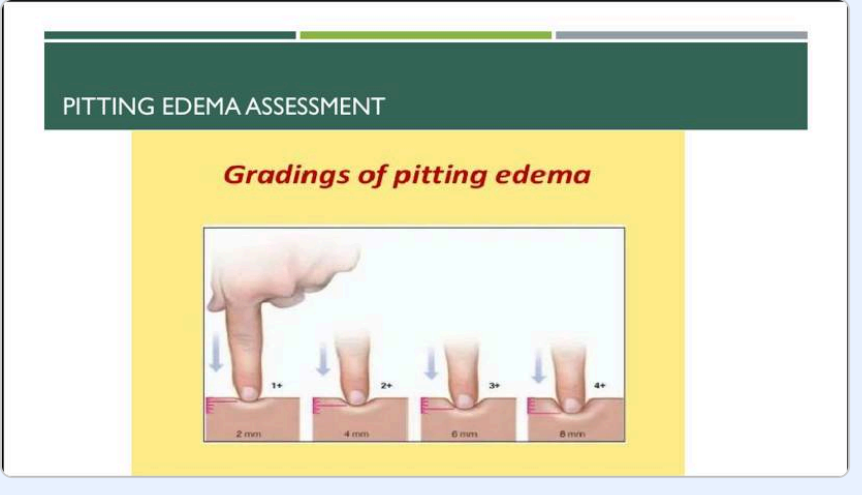


Where would you assess turgor? [Slide 14]

[Slide 15] EDEMA

- Edema pitting vs non pitting.
- If pitting, grade on scale: **1+** Mild pitting; **2+** Moderate pitting; **3+** Deep pitting; **4+** Very deep pitting.

[Slide 16] PITTING EDEMA ASSESSMENT — demonstration/photo of the technique.



Pitting edema assessment [Slide 16]

Jarvis fill-in [Jarvis pp. 209–211]:

- **Temperature** — skin should be warm and equal bilaterally (warmth suggests normal circulatory status); hands/feet may be slightly cooler in a cool environment. Localized coolness: immobilized extremity, arterial insufficiency, Raynaud disease. Generalized hyperthermia: fever, heavy exercise. Localized hyperthermia: trauma, infection, sunburn. Chronic smoking → cool, pale hands.
- **Moisture** — normal perspiration on face, hands, axillae, skinfolds with activity, warmth, anxiety. **Diaphoresis** (profuse) with increased metabolic rate, fever, perimenopause; abnormal with thyrotoxicosis, heart attack, anxiety, pain. Check oral mucous membranes for dehydration (normally smooth and moist; with dehydration, dry, lips parched and cracked). Dark skin may normally look dry and flaky without systemic dehydration.
- **Texture** — normally smooth and firm with an even surface. Hyperthyroidism: smoother/softer, like velvet. Hypothyroidism: rough, dry, flaky.
- **Thickness** — epidermis uniformly thin except calluses on palms/soles (normal adaptation to pressure/friction). Very thin, shiny (atrophic) skin with arterial insufficiency.
- **Mobility and turgor** — pinch up a large fold of skin on the **anterior chest under the clavicle** [Jarvis p. 210]; mobility = ease skin rises; turgor = ability to return promptly (elasticity). Poor turgor: skin recedes slowly or "tents" — severe dehydration or extreme weight loss. Mobility decreased with edema; scleroderma = decreased mobility.
- **Vascularity/bruising** — cherry (senile) angiomas are normal: small (1–5 mm), smooth, slightly raised bright red dots on the trunk in adults older than 30. Bruising should be consistent with expected trauma. Multiple bruises at different healing stages or bruises above knees/elbows raise concern about physical abuse. Document tattoos; inspect for infection/inflammation [Jarvis p. 211].

Edema technique [Jarvis p. 210]: imprint thumbs firmly for 3–4 seconds against the ankle malleolus or tibia, then lift; normally skin stays smooth; a dent = pitting edema. Edema is not present normally; it appears in dependent parts (feet, ankles, sacrum) where skin looks puffy and tight. Edema masks normal skin color and

makes dark skin look lighter. Unilateral edema = local/peripheral cause; bilateral or generalized (anasarca) = consider central cause (heart or kidney failure).

Pitting grades [Jarvis p. 518–519; matches Slide 15 wording]:

Grade	Jarvis description
1+	Mild pitting, slight indentation, no perceptible swelling of the leg
2+	Moderate pitting, indentation subsides rapidly
3+	Deep pitting, indentation remains a short time, leg looks swollen
4+	Very deep pitting, indentation lasts a long time, leg grossly swollen and distorted

Technique/reliability note [Jarvis pp. 518–519–520]: depress skin over the tibia or medial malleolus firmly for **5 seconds** and release (her handout/sheet cites p. 518–519 for pitting). This classic pit-depth method is commonly used but has **not been proven objective, reliable, or sensitive**; pressure used is arbitrary. **Ankle circumference is more reliable**, using non-stretchable tape 7 cm proximal to the midpoint of the medial malleolus. Conform to your institution's consistently used scale. Bilateral dependent pitting edema occurs with heart failure, diabetic neuropathy, hepatic cirrhosis; unilateral edema with deep vein occlusion; lymphatic obstruction gives "brawny" nonpitting, hard edema. With bilateral pitting edema, examine the neck veins.

> Her version is the short four-word scale (mild / moderate / deep / very deep) [Slide 15]; Jarvis gives the same four grades plus the added descriptions and the reliability caveat above [Jarvis pp. 518–519–520]. Dependent pallor/rubor and venous filling tests are also on pp. 520 (color return to feet normally ≤10 seconds; superficial vein filling about 15 seconds).

8. Describing lesions

Slide 17: Inspect & palpate the skin: lesions [Slide 17]

- Lesions — what are they? (changes in previously normal skin [Jarvis p. 211])
- If any are present note: **color**; **elevation**; **pattern or shape**; **size**; **location and distribution on body**; **any exudate: note color and odor**.

Handout's fuller documentation list [Lesion handout] — her slide list is the core; the handout adds these items:

- Number of lesions (actual count, or estimation)
- Location/distribution — be specific (scalp, face, trunk = high sebaceous gland concentration; on extremities note extensor vs flexor surface)
- Grouping or configuration (e.g., linear, annular)
- Size in centimeters
- Shape and borders (round, linear, oval, geographic; borders well-defined, ill-defined, circumscribed, irregular)
- Surface (flat or raised)
- Tenderness (or non-tender)
- Mobility (mobile = moves with the skin; non-mobile/fixed = skin moves over it)
- Special characteristics (e.g., pediculated on a stalk, exudative)

Jarvis agrees with the slide checklist and adds technique [Jarvis p. 211-212]: measure with a ruler — avoid "quarter size"/"pea size"; palpate lesions (glove if contact with blood/mucosa/fluid); roll a nodule between thumb and index finger for depth; gently scrape a scale with a tongue blade; note the base and whether it bleeds; check surrounding skin temperature; ask: **does it blanch with pressure or stretch?** (red macules from dilated vessels blanch; extravasated blood such as petechiae does not).

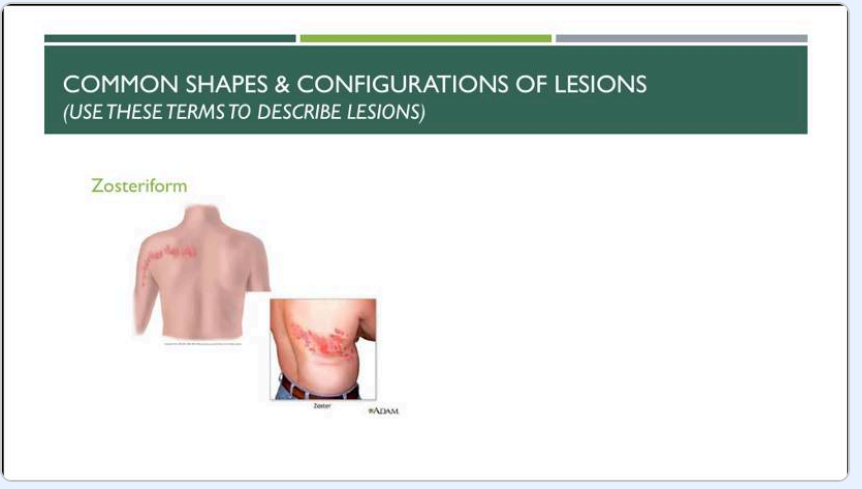
Describe a lesion	What to note
Color	Hue; uniform or varied [Slide 17; Lesion handout]
Number	Actual count or estimation [Lesion handout]
Elevation / surface	Flat, raised, or pedunculated [Slide 17; Jarvis p. 211]
Pattern / configuration	Grouping or distinctness of each lesion (e.g., annular, grouped, confluent, linear) [Slide 17; Jarvis p. 211]
Shape & borders	Round, linear, oval, geographic; well-defined, ill-defined, circumscribed, irregular [Lesion handout]
Size	Centimeters, measured with a ruler [Slide 17; Jarvis p. 211]
Location & distribution	Generalized vs localized; body site, be specific [Slide 17; Lesion handout]
Exudate	Color and odor [Slide 17; Jarvis p. 211]
Tenderness / mobility	Tender or non-tender; mobile vs fixed [Lesion handout]
Special characteristics	Pediculated (on a stalk), exudative, etc. [Lesion handout]

> Documentation sheet wording matches this list: Type, Color, Pattern/Shape (Table 13-2, p. 228–229), Size, Location/Distribution, Exudate, Elevation [Module 3 Documentation Sheet].

9. Shapes and configurations

Common shapes & configurations of lesions — use these terms to describe lesions [Slides 18–21]

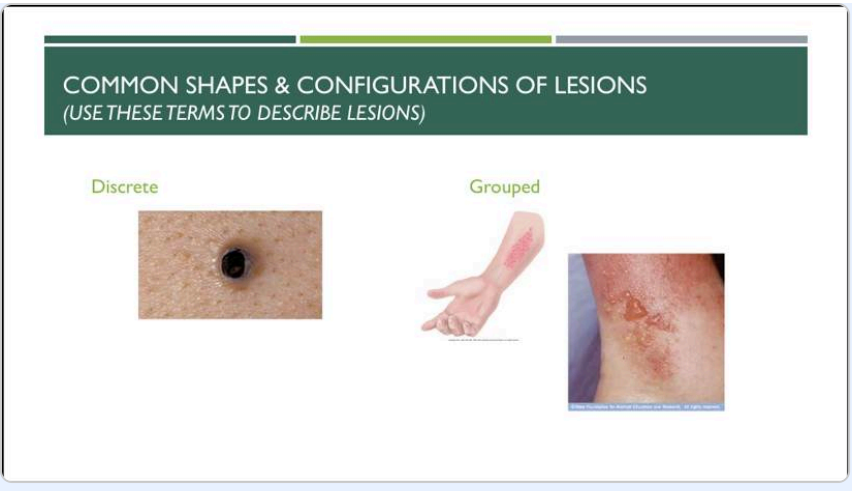
- Annular/circular [Slide 18] · Confluent [Slide 18]
- Discrete [Slide 19] · Grouped [Slide 19]
- Linear [Slide 20] · Target [Slide 20]
- Zosteriform [Slide 21]



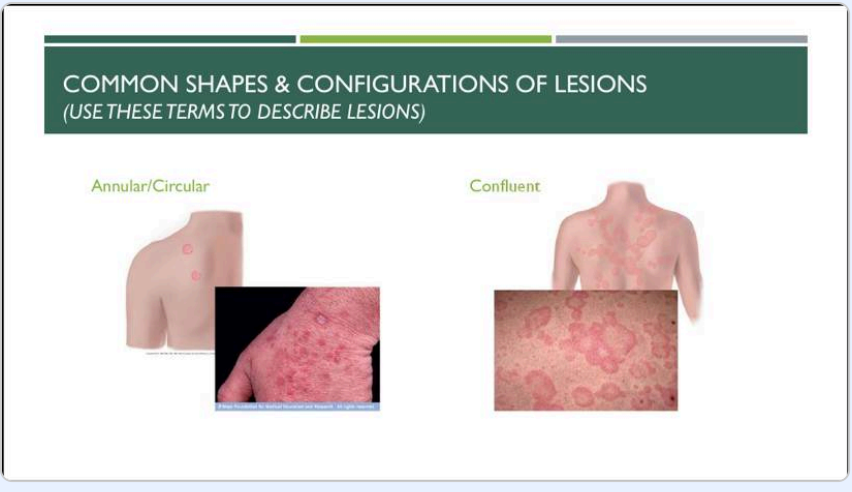
Shape: zosteriform [Slide 21]



Shapes: linear, target [Slide 20]



Shapes: discrete, grouped [Slide 19]



Shapes: annular, confluent [Slide 18]

Slides 18–21 show paired photos of each configuration (image slides; terms only).

Configuration = the shape or outline of lesions [Lesion handout]:

- **Annular** — round or circular with central clearing; **circinate** — round, circular > arciform (partial circle)
- **Iris / target** — concentric rings, dark or blistered center
- **Gyrate** — connecting arcs; **linear** — straight; **serpiginous** — meandering, like a snake's track
- **Margination** — sharp or ill-defined; **satellite lesions** — e.g., cutaneous candidiasis: beefy red plaque surrounded by smaller red macules adjacent to the main lesion
- **Zosteriform** — dermatomal

Distribution = the clinical arrangement of lesions [Lesion handout]: localized (grouped in specific areas); generalized (dispersed all over); symmetric (no pattern); asymmetric (pattern lacking randomness); discrete (separate); grouped (clustered); confluent/coalescing (smaller into larger); cleavage plane (arranged along lines of skin tension).

Term	Jarvis definition (Table 13.2)	Example
Annular / circular	Begins in center, spreads to periphery	Tinea corporis, tinea versicolor, pityriasis rosea
Confluent	Lesions run together	Urticaria (hives)
Discrete	Distinct, individual, remain separate	Skin tags, acne
Grouped	Clusters of lesions	Vesicles of contact dermatitis
Linear	Scratch, streak, line, or stripe	—
Target / iris	Resembles iris of eye, concentric rings of color	Erythema multiforme
Zosteriform	Linear arrangement along a unilateral nerve route	Herpes zoster

Jarvis Table 13.2 also lists [Jarvis p. 229]: **gyrate** (twisted, coiled spiral, snakelike) and **polycyclic** (annular lesions grow together, e.g., lichen planus, psoriasis) — her slides show gyrate only via the handout, and polycyclic only in Jarvis. Her slide 18 pairs "annular/circular," matching Jarvis [Slides 18-21; Jarvis p. 228-229].

10. Primary lesions



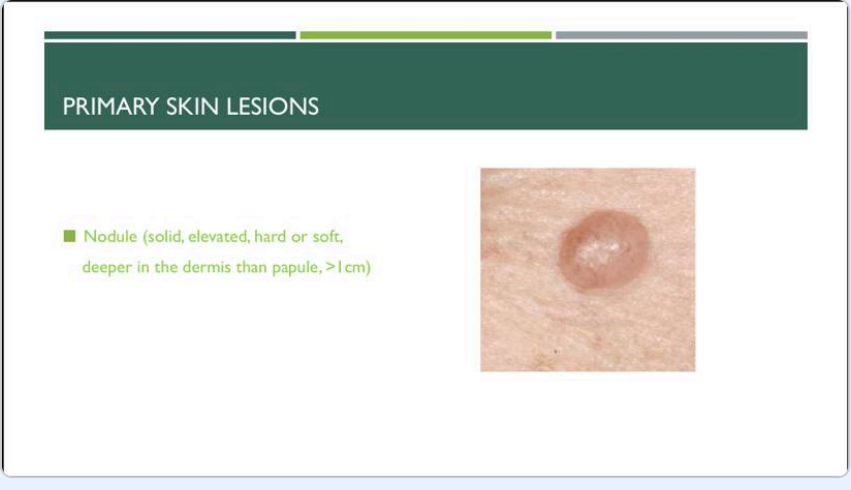
Skin Lesion Reference Guide photos (her handout) [Lesion handout p. 1]

Primary skin lesions [Slides 22–25]

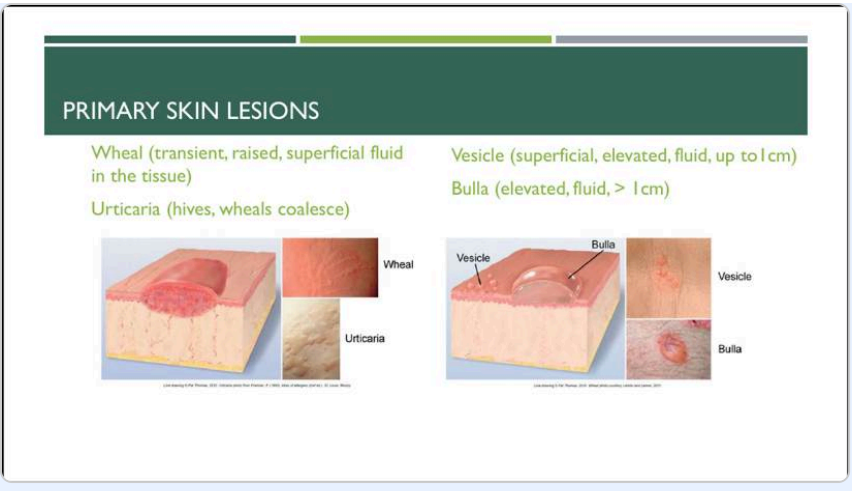
- Macule (flat, <1 cm) · Patch (macules, >1 cm) · Papule (elevated, <1 cm) · Plaque (papules join, >1 cm) [Slide 22]
- Wheal (transient, raised, superficial fluid in the tissue) · Vesicle (superficial, elevated, fluid, up to 1 cm) · Bulla (elevated, fluid, >1 cm) · Urticaria (hives, wheals coalesce) [Slide 23]
- Nodule (solid, elevated, hard or soft, deeper in the dermis than papule, >1 cm) [Slide 24]
- Cyst (fluid-filled cavity, deeper in dermis or subcutaneous layer) · Pustule (pus in cavity, circumscribed and elevated) [Slide 25]



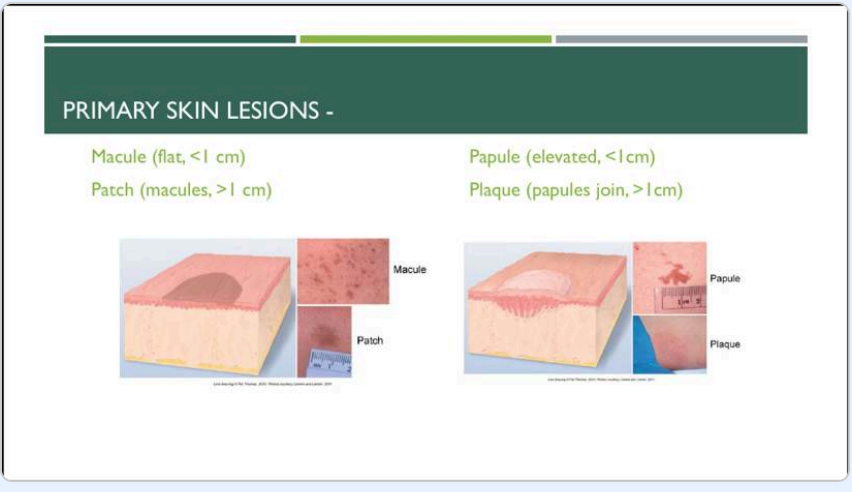
Primary: cyst, pustule [Slide 25]



Primary: nodule [Slide 24]



Primary: wheal, urticaria, vesicle, bulla [Slide 23]



Primary: macule, patch, papule, plaque [Slide 22]

⚠ **Size conflict — papule:** her slide says papule <1 cm [Slide 22] and Jarvis says less than 1 cm diameter [Jarvis p. 230], but the lesion handout says **≤0.5 cm** [Lesion handout]. Learn **<1 cm** as the class/lecture definition and ask her which cutoff she wants documented.

> Primary lesions are the immediate results of a specific causative factor and develop on previously unaltered skin [Jarvis p. 211, 230].

Lesion	Her definition	Example (Jarvis)
Macule	Flat color change, <1 cm [Slide 22]; handout: circular flat discoloration <1 cm, brown/blue/red/hypopigmented	Freckles, flat nevi, petechiae, measles
Patch	Macules >1 cm [Slide 22]	Vitiligo, café au lait spot, measles rash
Papule	Elevated, <1 cm [Slide 22] (handout: superficial solid, ≤0.5 cm)	Elevated nevus, wart, molluscum
Plaque	Papules join, >1 cm [Slide 22]	Psoriasis, lichen planus
Nodule	Solid, elevated, hard or soft, deeper than papule, >1 cm [Slide 24]	Xanthoma, fibroma, intradermal nevi

Lesion	Her definition	Example (Jarvis)
Wheal	Transient, raised, superficial fluid in tissue [Slide 23]; handout: edematous transitory plaque, may last hours	Mosquito bite, allergic reaction, dermographism
Urticaria	Hives; wheals coalesce [Slide 23]	Intensely pruritic extensive reaction
Vesicle	Superficial, elevated, fluid, up to 1 cm [Slide 23]	Herpes simplex, varicella, herpes zoster, contact dermatitis
Bulla	Elevated, fluid, >1 cm [Slide 23]; handout: circumscribed free fluid >1 cm	Friction blister, pemphigus, burns

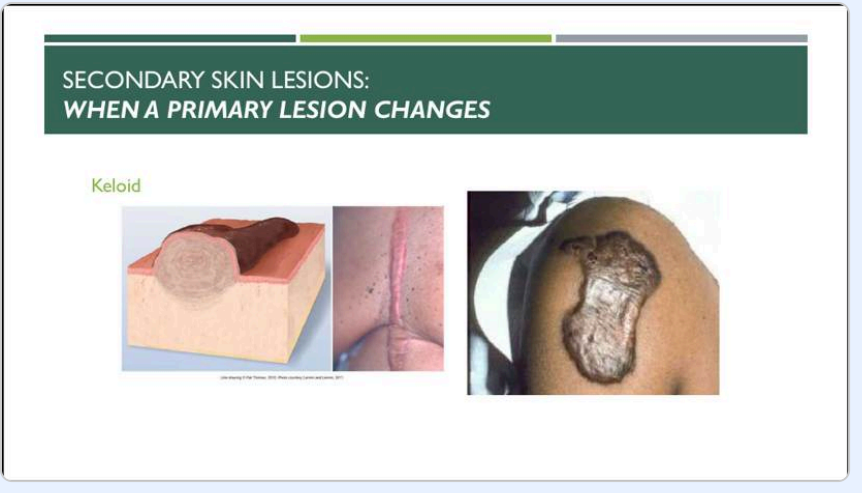
Lesion	Her definition	Example (Jarvis)
Cyst	Fluid-filled cavity, deeper in dermis or subcutaneous layer [Slide 25]	Sebaceous cyst, wen
Pustule	Pus in cavity, circumscribed and elevated [Slide 25]; handout: vesicle containing pus	Impetigo, acne
Tumor	Not on her slides	Lipoma, hemangioma (Jarvis only: larger than a few cm, firm or soft) [Jarvis p. 230]

> Jarvis also notes the bulla is usually single-chambered (unilocular), thin-walled, ruptures easily; a ruptured vesicle gives clear serum [Jarvis p. 230].

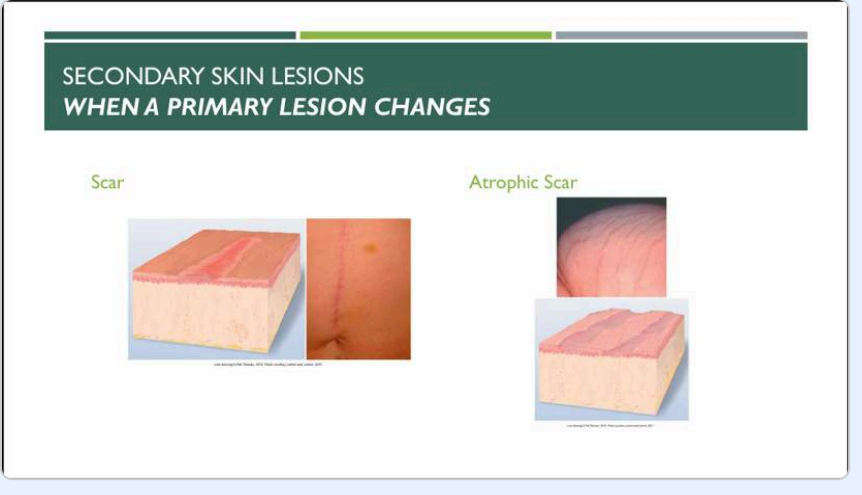
11. Secondary lesions

Secondary skin lesions: when a primary lesion changes [Slides 26–31]

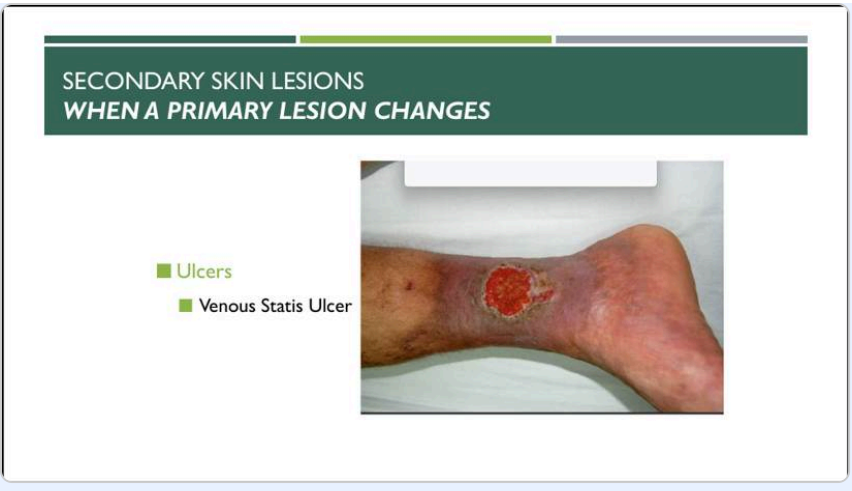
- Crust (dried-out exudate when vesicles/pustules burst) · Scale (compact, dry flakes of skin) [Slide 26]
- Fissure (linear crack) · Erosion (shallow loss of epidermis) [Slide 27]
- Ulcer (deeper depression extending into dermis, leaves scar when heals) [Slide 28]
- Ulcers — venous stasis ulcer [Slide 29]
- Scar · Atrophic scar [Slide 30]
- Keloid [Slide 31]



Secondary: keloid [Slide 31]



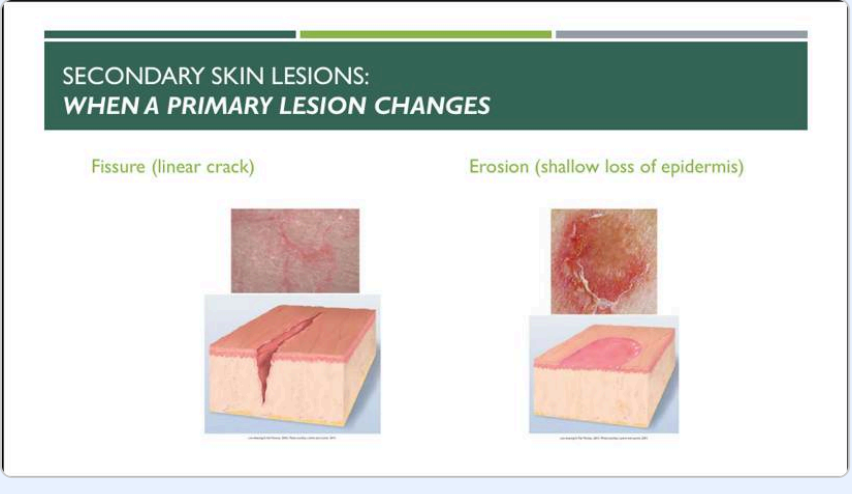
Secondary: scar, atrophic scar [Slide 30]



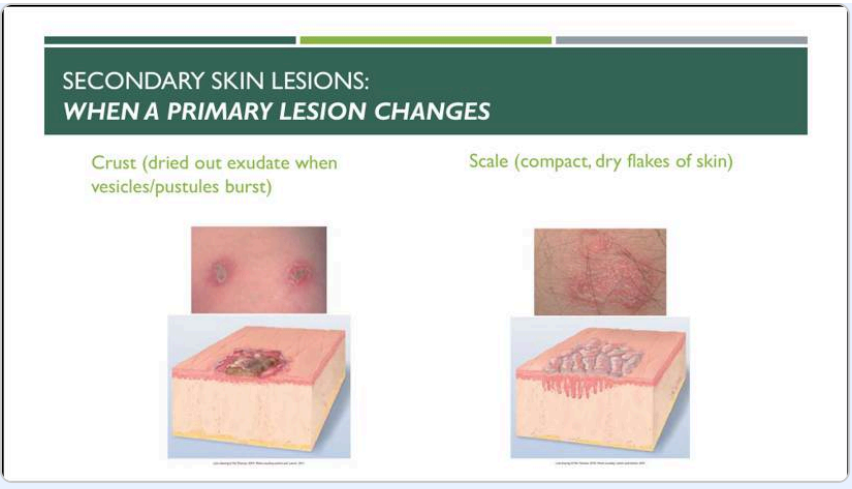
Secondary: venous stasis ulcer [Slide 29]



Secondary: ulcer [Slide 28]



Secondary: fissure, erosion [Slide 27]



Secondary: crust, scale [Slide 26]

Secondary lesions result from change in a primary lesion over time; primary and secondary lesions may coexist (papulosquamous, maculopapular, vesiculopustular, papulovesicular) [Jarvis p. 231]. Handout adds two terms not on her slides [Lesion handout]: **excoriation** (linear erosion) and **lichenification** (thickening of the epidermis with exaggeration of normal skin lines).

Lesion	Definition	Example (Jarvis Table 13.4)
Crust	Dried-out exudate after vesicles/pustules burst; red-brown, honey, or yellow [Slide 26]	Impetigo (honey-colored), eczematous dermatitis, scab
Scale	Compact dry/greasy flakes of dead keratin [Slide 26]	Psoriasis (silver, mica-like), seborrheic dermatitis (greasy yellow)
Fissure	Linear crack with abrupt edges, extends into dermis [Slide 27]	Cheilosis, athlete's foot
Erosion	Shallow, scooped-out; epidermis lost; heals without scar [Slide 27]	—

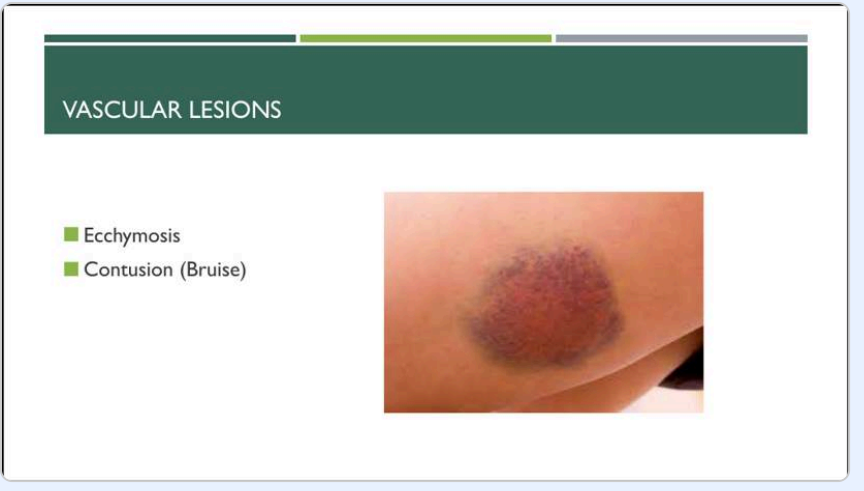
Lesion	Definition	Example (Jarvis Table 13.4)
Ulcer	Deeper depression into dermis, irregular; may bleed; leaves scar [Slide 28]	Stasis ulcer, pressure injury, chancre
Excoriation	Self-inflicted superficial abrasion from scratching [Lesion handout; Jarvis p. 232]	Insect bites, scabies, varicella
Scar	Permanent fibrotic change; tissue replaced by collagen [Slide 30]	Healed surgery/injury, acne
Atrophic scar	Depressed skin level with loss of tissue, epidermis thinned [Slide 30]	Striae

Lesion	Definition	Example (Jarvis Table 13.4)
Lichenification	Thickened epidermis with exaggerated skin lines; from prolonged scratching [Lesion handout]	Looks like the surface of moss
Keloid	Benign excess scar tissue beyond original injury; smooth, rubbery, shiny, "clawlike" [Slide 31]	Surgery, acne, ear piercing, tattoos, burns; ages 10–30; higher incidence in Black, Latino, Asian people
Venous stasis ulcer	Ulcer type named on her slide [Slide 29]	Stasis ulcer (Jarvis) — dusky rubor of dependent extremities precedes necrosis [Jarvis p. 227]

12. Vascular lesions

Vascular lesions [Slides 32–35]

- Hemangiomas — port-wine stain, strawberry mark, cherry angioma; telangiectases — spider or star angioma [Slide 32]
- Port wine stain · Strawberry mark (photos) [Slide 33]
- Petechiae · Purpura (photos) [Slide 34]
- Ecchymosis · Contusion (bruise) [Slide 35]



Vascular: ecchymosis (bruise) [Slide 35]



Vascular: petechiae, purpura [Slide 34]



Vascular: port-wine stain, strawberry mark [Slide 33]



Vascular lesions overview [Slide 32]

Jarvis Table 13.6 groups these as: hemangiomas (benign proliferation of blood vessels in the dermis); telangiectases (permanently enlarged/dilated vessels visible on the surface); purpuric lesions (blood outside vessel walls — extravascular, so **do not blanch**; hard to see in dark-skinned people) [Jarvis pp. 235-237]. Cherry (senile) angiomas: small 1–5 mm bright red dots on the trunk, all adults >30 years, increase with aging, not significant [Jarvis p. 211].

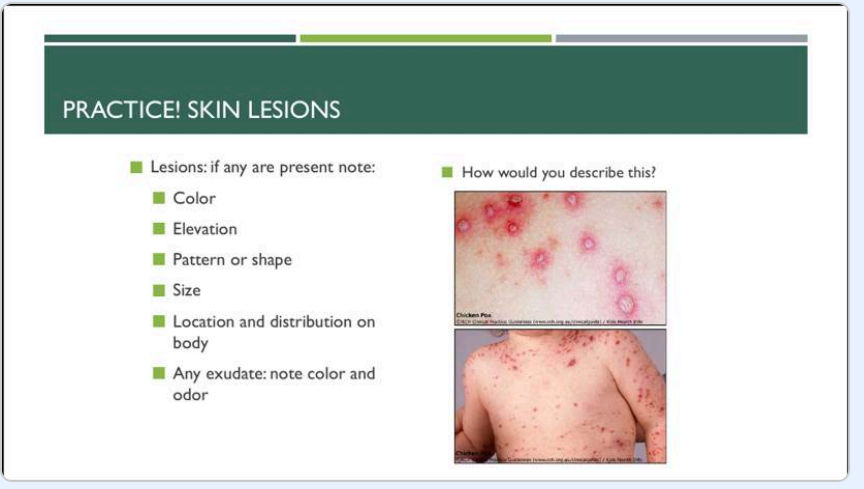
Lesion	Size / blanching	Cause / notes
Port-wine stain	Large flat macular patch; does not state blanching	Mature capillaries; present at birth, usually does not fade; scalp/face, cranial nerve V
Strawberry mark	Raised, about 2–3 cm; **does not blanch**	Benign endothelial growth; neonatal; most involute by age 3
Spider telangiectasia	Fiery red, star-shaped; central body blanches, legs blanch with pressure	Pregnancy, chronic liver disease, estrogen therapy — or normal
Venous lake	Blue-purple; pressure empties/disappears	Lips, ears, face of older adults

Lesion	Size / blanching	Cause / notes
Petechiae	1–3 mm, round, discrete; **will not blanch**	Bleeding from superficial capillaries; may indicate abnormal clotting; inspect buccal mucosa & conjunctivae
Purpura	>3 mm, flat, red-purple; does not blanch	Confluent petechiae and ecchymoses; thrombocytopenia, coagulation disorders, scurvy, old age
Ecchymosis	Purplish patch, >3 mm	Extravasation of blood into skin
Contusion (bruise)	>3 mm; pressure **does not blanch**	Mechanical injury (blow); skin intact. Note: **cannot date a bruise by color** [Jarvis p. 237]

> Jarvis's distinction to remember: a bruise differs from petechiae, ecchymosis, and purpura because those three are **not** caused by blunt force trauma [Jarvis p. 237]. Color progression in light skin: red-blue/purple → blue-purple → blue-green → yellow → brown (not for dating) [Jarvis p. 237]. Multiple bruises at different stages of healing, or bruises above knees/elbows, raise concern about physical abuse [Jarvis p. 211].

13. Practice: describing lesions

Practice! Skin lesions [Slides 36–37] — image slides ("How would you describe this?") with the same checklist repeated: color; elevation; pattern or shape; size; location and distribution on body; exudate (color and odor).



Practice: describe this lesion [Slide 37]



Practice: describe this lesion [Slide 36]

Step-by-step description template (her checklist + handout items) [Slide 36-37; Lesion handout; Jarvis p. 211]:

1. **Number** — count or estimate.
2. **Type** — primary or secondary (macule, papule, plaque, vesicle, crust…).
3. **Color** — one word; note variation.

4. **Elevation / surface** — flat, raised, pedunculated.
5. **Pattern or shape** — annular, grouped, confluent, linear, target, zosteriform, discrete.
6. **Size** — centimeters, measured with a ruler (never "pea size").
7. **Location & distribution** — specific body site; generalized vs localized.
8. **Borders & palpation** — well/ill-defined; tender? mobile or fixed? blanches?
9. **Exudate** — color and odor.

Worked example 1 — using only Jarvis's petechiae description [Jarvis p. 237]: "Multiple (count ___) tiny punctate, round, discrete lesions, 1 to 3 mm, dark red (purple/brown), flat, non-blanching with pressure, located on the ___ (Jarvis notes buccal mucosa, conjunctivae, abdomen, buttocks, volar forearm as sites where they are best seen). No exudate." Read: abnormal clotting factors / thrombocytopenia / septicemias.

Worked example 2 — using only Jarvis's psoriasis plaque description [Jarvis p. 230-231]: "Grouped/confluent papules coalescing into plateaulike, disk-shaped surface elevations wider than 1 cm (measured ___ cm × ___ cm), located on the ___, bordered ___, with silvery, mica-like scale on the surface (scraped gently with tongue blade; base noted). Tenderness: ___." Psoriasis is also given by Jarvis as an example of polycyclic configuration [Jarvis p. 229].

14. Skin tears

Skin tears [Slides 38–39]

- A traumatic wound caused by mechanical forces, including removal of adhesives [Slide 38]
- Loss of collagen with aging increases the risk [Slide 38]
- **Handle with care!** [Slide 38]
- Slide 39 is a photo slide of skin tears (image only) [Slide 39].



Skin tears [Slide 39]

Relates to Jarvis's finding of thin, fragile skin and decreased mobility with aging/edema [Jarvis p. 210]; no separate Jarvis section on skin tears in the assigned pages — learn her slide definition.

15. Pressure injuries

TABLE 13.5 Pressure Injuries (PI)

PIs appear on the skin over a bony prominence when circulation is impaired, e.g., when confined to bed or immobilized. Immobilization impedes delivery of blood carrying oxygen and nutrients to the skin, and it impedes venous drainage carrying metabolic wastes away from the skin. This results in ischemia and cell death. Common sites for PIs are on the back (heel, ischium, sacrum, elbow, scapula, vertebra) or the side (ankle, knee, hip, rib, shoulder).

Risk factors for PIs include impaired mobility, thin fragile skin of aging, decreased sensory perception (thus unable to respond to pain accompanying prolonged pressure), impaired level of consciousness (also unable to respond), moisture from urine or stool incontinence, excessive perspiration or wound drainage, shearing injury (being pulled down or across in bed), poor nutrition, and infection. Vasopressor administration (norepinephrine) increases risk of PI in critically ill patients because it targets alpha receptors.[7] Knowledge of risk factors and prevention of PIs is far more easily accomplished than is treatment of existing ulcers. However, once PIs occur, they are assessed by stage, depending on the pressure ulcer depth.[14] Once stage 3 or 4 ulcers occur, measure wound size daily. Use disposable rulers with mm and cm markings, and measure the greatest overall wound length and width.



Stage 1—Non-Blanchable Erythema

Intact skin is red but unbroken. Localized redness in lightly pigmented skin does not blanch (turn light with fingertip pressure). Dark skin appears darker but does not blanch. May have changes in sensation, temperature, or firmness.



Stage 2—Partial-Thickness Skin Loss

Loss of epidermis and exposed dermis. Superficial ulcer looks shallow like an abrasion or open blister with a red-pink wound bed. No visible fat or deeper tissue.



Stage 3—Full-Thickness Skin Loss

PI extends into subcutaneous tissue and resembles a crater. See subcutaneous fat, granulation tissue, and rolled edges, but not muscle, bone, or tendon.



Stage 4—Full-Thickness Skin/Tissue Loss

PI involves all skin layers and extends into supporting tissue. Exposes muscle, tendon, or bone, and may show slough (stringy matter attached to wound bed) or eschar (black or brown necrotic tissue), rolled edges, and tunneling.

Deep Tissue Pressure Injury (DTPI)

Localized, non-blanchable color change to deep red, maroon, purple in intact or nonintact skin. Dark skin appears darker but does not blanch. Or, epidermis may separate, revealing dark wound or blood-filled blister.[14] Preceded by pain and temperature change. Begins in the muscle closest to the bone, in older adults and those with a lower BMI, commonly on skin over coccyx, sacrum, buttocks, heels.

PI Caused by Medical Device

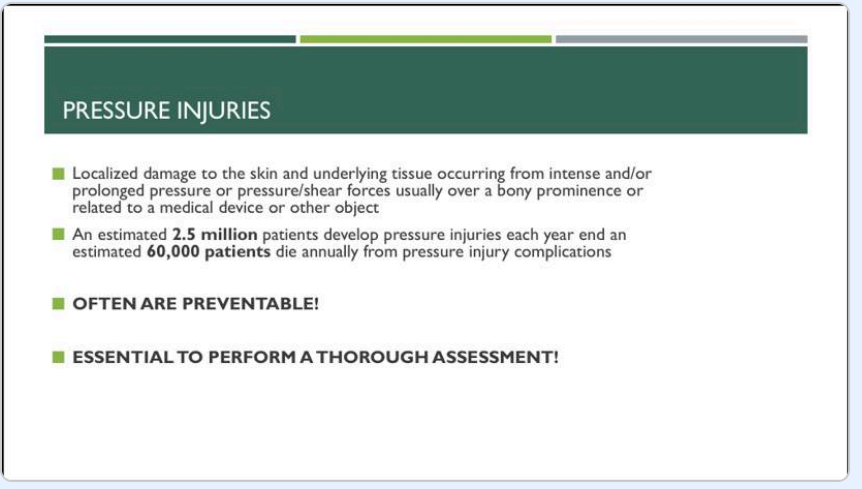
Skin or mucosa has PI that looks like pattern or shape of medical device (e.g., O_2 tubing, airway pressure masks, IV hub, endotrachial tube, cervical collar, sequential compression devices). Skin PI should be staged; mucosal PI cannot be staged. Rigid plastic of medical device is necessary to maintain shape and function of object but causes pressure on underlying skin and mucosa.[6]

See Illustration Credits for source information.

Abnormal Findings

Table 13.5 Pressure injuries [Jarvis p. 234]

Pressure injuries [Slide 40]



Pressure injuries [Slide 40]

- Localized damage to the skin and underlying tissue from intense and/or prolonged pressure or pressure/shear forces, usually over a bony prominence, or related to a medical device or other object.
- An estimated **2.5 million** patients develop pressure injuries each year and an estimated **60,000** patients die annually from pressure injury complications.
- OFTEN ARE PREVENTABLE!
- ESSENTIAL TO PERFORM A THOROUGH ASSESSMENT!

Jarvis fill-in [Jarvis p. 234]: PIs appear over a bony prominence when circulation is impaired (bed/chair confinement); immobilization impedes oxygen/nutrient delivery and venous drainage → ischemia and cell death. Common sites: back (heel, ischium, sacrum, elbow, scapula, vertebra) and side (ankle, knee, hip, rib, shoulder). Risk factors: impaired mobility, thin fragile aging skin, decreased sensory perception, impaired LOC, moisture (urine/stool incontinence, perspiration, wound drainage), shearing injury (pulled down/across in bed), poor nutrition, infection; vasopressors (norepinephrine) increase risk in critically ill. Prevention is far easier than treatment. Once stage 3 or 4 ulcers occur, **measure wound size daily** with disposable mm/cm rulers — greatest overall length and width.

Stage	What you see
Stage 1 — non-blanchable erythema	Intact but red, unbroken skin; does not blanch with fingertip pressure; dark skin appears darker but does not blanch; may have changes in sensation, temperature, firmness
Stage 2 — partial-thickness skin loss	Epidermis lost, dermis exposed; shallow abrasion or open blister look, red-pink wound bed; no visible fat or deeper tissue
Stage 3 — full-thickness skin loss	Crater extending into subcutaneous tissue; subcutaneous fat, granulation tissue, rolled edges visible; not muscle, bone, or tendon
Stage 4 — full-thickness skin/tissue loss	All skin layers plus supporting tissue; muscle, tendon, or bone exposed; may show slough or eschar, rolled edges, tunneling

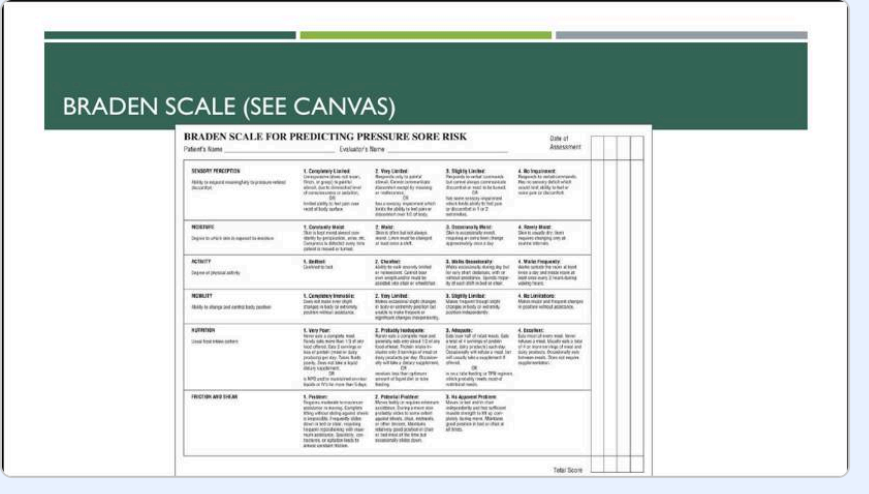
Stage	What you see
Deep tissue pressure injury (DTPI)	Localized non-blanchable deep red, maroon, purple on intact or nonintact skin; dark skin darker, does not blanch; or epidermis separates revealing dark wound or blood-filled blister; preceded by pain and temperature change; often coccyx, sacrum, buttocks, heels
Device-related PI	Looks like the pattern/shape of the device (O_2 tubing, airway mask, IV hub, ET tube, cervical collar, SCDs); **skin PI is staged; mucosal PI cannot be staged**

> Slide 40 uses "pressure injuries"; Jarvis Table 13.5 uses "pressure injuries (PI)" but in the text of Table 13.4 uses "pressure injury" as an ulcer example [Slide 40; Jarvis p. 232-234]. Common sites on the slide = "usually over a bony prominence" [Slide 40].

16. Braden Scale

Braden Scale for Predicting Pressure Sore Risk [Slides 41–45]

- **Six subscales**: Sensory Perception; Moisture; Activity; Mobility; Nutrition; Friction & Shear [Slide 41]
- Braden Scale (see Canvas) [Slide 42]
- **Score bands** [Slide 43]: Mild risk 15–18; Moderate 13–14; High 10–12; Severe risk <9
- Case study: Mr. Gupta, 82-year-old male, status post stroke, admitted to inpatient Rehabilitation Center at Loma Linda East Campus Hospital — prompts for each subscale [Slide 44]
- What is Mr. Gupta's Braden score? Risk level? Fill in on the Module 3 Documentation Sheet (Score: ____; Risk Level: ____) [Slide 45] — *class exercise; do not pre-fill.*



Braden Scale (see Canvas) [Slide 42]

⚠ **Score-band conflict:** slide 43 says severe risk **<9** [Slide 43], but her Braden handout says **SEVERE ≤9** [Braden handout]. Learn **≤9** from the handout and ask her which one she wants on the exam. Handout also states: "Total score of 12 or less represents HIGH RISK." **Lower score = higher risk.** Total range **6–23**.

Subscale	1	4
Sensory perception (ability to respond meaningfully to pressure-related discomfort)	Completely limited — unresponsive to painful stimuli (diminished consciousness/sedation) or limited ability to feel pain over most of body	No impairment
Moisture (degree skin exposed to moisture)	Constantly moist — perspiration/urine; damp every time moved or turned	Rarely moist — linen at routine intervals
Activity	Bedfast	Walks frequently — outside room at least twice a day, inside room at least every 2 hours while awake
Mobility (ability to change and control body position)	Completely immobile	No limitations
Nutrition (usual food intake)	Very poor — never eats a complete meal, rarely >1/3 of food, ≤2 servings protein/day, poor fluids, no supplement; or NPO/clear liquids/IV >5 days	Excellent — most of every meal, never refuses, 4+ servings protein, eats between meals, no supplementation needed
Friction & shear	Problem — moderate to maximum assistance moving; lifting without sliding impossible; frequently slides down; spasticity/contractures/agitation → constant friction	**No apparent problem (3)** — this subscale runs 1–3, maximum 3 [Braden handout]

> **Friction & shear note:** it is scored 1–3 only (1 Problem, 2 Potential problem — moves feebly or needs minimum

assistance, skin probably slides some, 3 No apparent problem). All other subscales run 1–4. Hence total range 6–23 [Braden handout].

Risk bands [Braden handout; Slide 43]: Severe ≤9 (slide: <9) · High 10–12 · Moderate 13–14 · Mild 15–18.

Mr. Gupta — questions to ask per subscale (do NOT compute his score; that's the class exercise) [Slide 44]:

- **Sensory perception** — any impairment? (post-stroke: can he feel/respond to pressure-related discomfort? respond to verbal commands? communicate discomfort?)
- **Moisture** — continent of bowel/bladder? diaphoresis? how often does the linen need changing?
- **Activity** — degree of physical activity? (bedfast / chairfast / walks occasionally / walks frequently)
- **Mobility** — able to change and control position on his own?
- **Nutrition** — ability to eat and nutritional status? (how much of each meal, protein servings, supplements, tube feeding/TPN?)
- **Friction & shear** — minimal/moderate/maximal assist when moving? does he slide down in bed/chair? spasticity, contractures, agitation?

Documentation sheet field: "Braden score: _____ Risk Level: _____ (Braden & Bergstrom, 1988)" [Module 3 Documentation Sheet].

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17. Reading focus: her assigned tables

Condensed cram versions of Jarvis Tables 13.1–13.5 and 13.10. Her slides introduce these topics [Slide 9–35]; the tables supply the detail. Where her wording and Jarvis differ (e.g., lesion size cutoffs), her version is listed first.

Table 13.1 — Detecting color changes in light and dark skin

[Jarvis p. 226–228]

Etiology	Light skin	Dark skin
Pallor — anemia, shock, vasoconstriction	Generalized pallor	Brown skin yellow-brown, dull; Black skin ashen gray, dull; loses healthy glow
Acute arterial insufficiency	Sudden marked localized pallor or mottling (e.g., lower legs), absent pulse	Ashen gray, dull; cool; sudden onset; absent pulses
Chronic arterial insufficiency	Pallor on elevation, rubor on dependency	Inspect for swelling, edema
Albinism / Vitiligo	Whitish pink / patchy milky-white spots, often symmetric	Tan, cream, white / same (patchy white)
Cyanosis (central or peripheral)	Dusky blue; peripheral = nail beds dusky	Dark but dull, lifeless; only severe cyanosis apparent — check conjunctivae, oral mucosa, nail beds
Erythema — hyperemia (inflammation, fever, blushing)	Red, bright pink	Purplish tinge, hard to see; palpate for warmth, taut skin, deep hardening
Polycythemia / Carbon monoxide	Ruddy blue face, mucosa, hands, feet / bright cherry red face, upper torso	Pigment conceals; check lips / cherry-red nail beds, lips, oral mucosa
Venous stasis	Dusky rubor of dependent extremities (prelude to necrosis)	Easily masked; use palpation for warmth or edema
Jaundice — high bilirubin	Yellow in sclera, hard palate, mucous membranes, then skin	Check sclera yellow up to iris (not normal fatty deposits); best at hard/soft palate junction, also palms

Etiology	Light skin	Dark skin
Carotenemia	Yellow-orange forehead, palms, soles, nasolabial folds; NO sclera/mucosa yellow	Yellow-orange tinge in palms and soles
Uremia / Addison / café au lait	Orange-green-gray over pallor / bronzed "eternal tan" / tan-brown oval patch, defined borders	Easily masked — rely on labs and clinical findings (uremia, Addison)

Table 13.2 — Common shapes and configurations of lesions [Jarvis p. 228–229; Slide 18–21]

Term	Meaning	Example
Annular (circular)	Begins in center, spreads to periphery	Tinea corporis, tinea versicolor, pityriasis rosea
Confluent	Lesions run together	Urticaria (hives)
Discrete	Distinct, individual, remain separate	Skin tags, acne
Gyrate	Twisted, coiled spiral, snakelike	—
Grouped	Clusters of lesions	Vesicles of contact dermatitis
Linear	Scratch, streak, line, or stripe	—
Target (iris)	Concentric rings of color	Erythema multiforme
Zosteriform	Linear along a unilateral nerve route (dermatome)	Herpes zoster
Polycyclic	Annular lesions grow together	Lichen planus, psoriasis

> [Lesion handout] adds configurations: circinate (partial circle), serpiginous (meandering), satellite lesions (candidiasis), marginal well-defined vs ill-defined; distribution terms: localized,

> generalized, symmetric, asymmetric, discrete, grouped, confluent (coalescing), cleavage plane.

Table 13.3 — Primary skin lesions [Jarvis p. 230–231; Slide

22–25; Lesion handout]

Lesion	Size	Definition	Examples
Macule	<1 cm	Flat, circumscribed, color change only	Freckle, flat nevus, petechiae, measles
Patch	>1 cm	Macules larger than 1 cm	Mongolian spot, vitiligo, café au lait
Papule	<1 cm (handout: ≤0.5 cm)	Solid, elevated, circumscribed	Elevated nevus, wart, lichen planus
Plaque	>1 cm	Papules coalesce; plateau, disk-shaped	Psoriasis, lichen planus
Nodule	>1 cm	Solid, elevated, hard or soft, deeper in dermis than papule	Xanthoma, fibroma
Tumor	Several cm	Firm or soft, deeper in dermis; may be benign or malignant	Lipoma, hemangioma
Wheal	Variable	Superficial, raised, transient, erythematous, edema, irregular shape	Mosquito bite, allergic reaction
Urticaria	—	Wheals coalesce; intensely pruritic	Hives
Vesicle	Up to 1 cm	Elevated cavity of free fluid; "blister"; clear serum if ruptured	Herpes simplex, chickenpox, shingles
Bulla	>1 cm	Usually single-chambered, thin-walled, ruptures easily	Friction blister, pemphigus, burns
Cyst	—	Encapsulated fluid-filled cavity in dermis/subcutaneous layer, tensely elevates skin	Sebaceous cyst, wen
Pustule	—	Pus (turbid fluid) in cavity; circumscribed, elevated	Impetigo, acne

Table 13.4 — Secondary skin lesions [Jarvis p. 231–233; Slide 26–31; Lesion handout]

Lesion	Definition	Examples / key point
Crust	Thickened dried-out exudate after vesicles/pustules burst	Red-brown, honey, or yellow; impetigo, scab
Scale	Compact dry/greasy flakes of dead keratin	Psoriasis (silver), seborrheic dermatitis (yellow greasy)
Fissure	Linear crack with abrupt edges, into dermis	Cheilosis, athlete's foot
Erosion	Shallow scooped-out depression; epidermis lost	Moist, no bleeding — **heals without scar**
Ulcer	Deeper depression into dermis, irregular, may bleed	Stasis ulcer, pressure injury, chancre — **leaves scar**
Excoriation	Self-inflicted abrasion, superficial, sometimes crusted	Scratches from itching: insect bites, scabies
Scar	Permanent fibrotic change; collagen replaces normal tissue	Healed surgery, acne
Atrophic scar	Depressed, thinned epidermis	Striae
Lichenification	Thickened skin from prolonged scratching; looks like moss	Chronic dermatitis
Keloid	Benign excess scar tissue **beyond original injury**; smooth, rubbery, shiny	Ear lobes, chest, back; ages 10–30; more in Black, Latino, Asian people

Table 13.5 — Pressure injuries [Jarvis p. 234; Slide 40–45]

Stage	Key findings	Depth / notes
Stage 1	Intact skin, non-blanchable redness; dark skin darker, does not blanch; may have sensation, temperature, firmness changes	Unbroken skin
Stage 2	Shallow open area/abrasion or blister, red-pink wound bed; no visible fat	Epidermis lost, dermis exposed
Stage 3	Crater; subcutaneous fat, granulation tissue, rolled edges; no muscle/bone/tendon	Full-thickness into subcutaneous tissue
Stage 4	Exposes muscle, tendon, or bone; slough, eschar, rolled edges, tunneling	All skin layers + supporting tissue
Deep tissue (DTPI)	Deep red, maroon, purple non-blanchable change; or blood-filled blister; preceded by pain and temperature change	Begins in muscle near bone; coccyx, sacrum, buttocks, heels
Medical device	Matches pattern/shape of device (O2 tubing, mask, IV hub, collar)	Skin PI staged; **mucosal PI cannot be staged**

> [Jarvis p. 234] Common sites: back (heel, ischium, sacrum, elbow, scapula, vertebra) and side (ankle, knee, hip, rib, shoulder). Risk factors: impaired mobility, thin fragile aging skin, decreased sensory perception, impaired LOC, moisture, shearing, poor nutrition, infection, vasopressors. Once stage 3 or 4: measure wound size daily with disposable rulers (greatest length and width). [Slide 40] ~2.5 million patients/year; ~60,000 deaths/year; often preventable.

Table 13.10 — Abnormal conditions of the hair [Jarvis p. 246; Slide 6–7]

Condition	Findings	Notes
Seborrheic dermatitis (cradle cap)	Thick yellow-to-white greasy adherent scales + mild erythema on scalp and forehead	Very common in early infancy; no pruritus; negative allergy history
Folliculitis barbae ("razor bumps")	Multiple pustules/"whiteheads" with hair at center, erythematous base; face and neck	Common in Black and Latino men with tight curly hair; hairs curl back into skin after shaving
Pediculosis capitis (head lice)	Nits translucent, teardrop, clear to light brown, adherent to hair shaft; intense pruritus	Preschool/school-age; check behind ears and nape of neck; no socioeconomic correlation

> Slide 7: photo of head lice (pediculosis capitis) — filled from Jarvis above [Slide 7; Jarvis p. 246].

18. Developmental considerations

Infants and children [Jarvis p. 235, 246, 207–208]

- Physiologic jaundice in the newborn is the one time jaundice occurs normally [Jarvis p. 209].
- Strawberry mark (infantile hemangioma) is present in the neonatal period, grows for the first few months, and most involute by age 3 years; complication is ulceration [Jarvis p. 235].
- Seborrheic dermatitis (cradle cap) is very common in early infancy [Jarvis p. 246].
- Junctional nevi are macular and occur in children and adolescents; they progress to compound nevi in young adults [Jarvis p. 207].
- Head lice are common in preschoolers and school-age children [Jarvis p. 246].
- Mongolian spot is an example of a patch in the young [Jarvis p. 230].

Older adults [Jarvis p. 211, 234, 237; Slide 38]

- Cherry (senile) angiomas occur in all adults older than 30 and increase in size and number with aging; not significant [Jarvis p. 211].
- Thin fragile skin of aging is a pressure injury risk factor [Jarvis p. 234].
- Loss of collagen with aging increases skin tear risk; handle with care [Slide 38].
- Purpura also occurs in old age as blood leaks from capillaries after minor trauma [Jarvis p. 237].
- Deep tissue pressure injuries begin in muscle closest to bone in older adults and those with lower BMI [Jarvis p. 234].
- Venous lakes appear on lips, ears, and face of older adults [Jarvis p. 236].

Skin tears [Slide 38–39]: a traumatic wound caused by mechanical forces including removal of adhesives; risk rises with age-related collagen loss. Handle with care!

19. Documentation examples

> **Example only — not your documentation sheet.** Generic findings in the order of the Module 3 documentation sheet. Do not copy onto the assignment.

Normal

- **Color:** Even/homogeneous, consistent with genetic background; no pallor, erythema, cyanosis, or jaundice.
- **Moles (ABCDEF):** Left upper back, 4 mm, symmetrical, borders well defined, uniform tan color, no elevation, no recent changes, not "funny looking."
- **Temperature:** Warm and equal bilaterally.
- **Moisture:** Slight perspiration; no diaphoresis or excessive dryness.
- **Texture:** Smooth and firm.
- **Edema:** None noted (no location to fill in).
- **Turgor:** Skin pinched under the clavicle returns immediately.
- **Bruising:** None noted.
- **Lesions:** None noted.
- **Braden score:** 20 — Risk Level: Mild risk.
- **Hair:** Color brown; texture smooth and shiny, straight.
- **Scalp:** Clean; no scales, lesions, growths, parasites, or loose white flakes.
- **Nail edges:** Smooth, rounded, clean.
- **Nail base angle:** 160°.
- **Consistency:** Smooth/regular; not brittle, splitting, pitted, grooved, or ridged.
- **Adherence:** Nail firmly adheres to nail bed.
- **Nail bed color:** Even pink.
- **Capillary refill:** <2 seconds.

Abnormal

- **Color:** Generalized pallor of conjunctivae and nail beds; no cyanosis or jaundice.
- **Moles (ABCDEF):** Upper back, 8 mm, asymmetrical, borders irregular/blurred, color variegated brown-black, elevation noted, recent change in size, "funny looking" compared with neighboring nevi [Jarvis p. 207–208].
- **Temperature:** Right lower leg cool compared with left.
- **Moisture:** Diaphoresis.
- **Texture:** Rough, dry, flaky.

- **Edema:** Location bilateral ankles; pitting 2+ moderate — indentation subsides rapidly [Jarvis p. 518–519].
- **Turgor:** Skin tents and recedes slowly (poor turgor).
- **Bruising:** Ecchymosis 3 cm on right forearm, deep purple.
- **Lesions:** Type: vesicle; color clear; pattern/shape: grouped; size 0.5 cm; location/distribution: left flank in a band (zosteriform); exudate none; elevated.
- **Braden score:** 11 — Risk Level: High risk.
- **Hair:** Color gray; texture dull, coarse, brittle.
- **Scalp:** Scales on scalp; loose white flakes (dandruff).
- **Nail edges:** Jagged, bitten.
- **Nail base angle:** ≥180° with spongy nail base (early clubbing) [Jarvis p. 214].
- **Consistency:** Brittle with pitting.
- **Adherence:** Nail lifted from nail bed.
- **Nail bed color:** Dusky.
- **Capillary refill:** >2 seconds (sluggish).

20. Quick-review tables

Key numbers

Number	What it means	Source
160°	Normal nail base angle (profile sign); curved nail is 160° or less	Slide 8; Jarvis p. 214
180°	Early clubbing — angle straightens, base feels spongy	Slide 8; Jarvis p. 214
1–2 s (assess <2 s)	Normal capillary refill; sluggish >3 s = clinical decline	Documentation sheet; Jarvis p. 215
1 cm	Vesicle ≤1 cm vs bulla >1 cm; macule <1 cm vs patch >1 cm	Slide 22–23; Jarvis p. 230
6 mm	Normal mole size; ABCDEF "D" diameter greater than 6 mm (pencil eraser)	Slide 12; Jarvis p. 208
3 mm	Purpura >3 mm; ecchymosis >3 mm; petechiae 1–3 mm	Jarvis p. 237
3–4 s / 5 s	Press thumbs 3–4 s for edema (Ch. 13); depress tibia 5 s (Ch. 21)	Jarvis p. 210, 519
1+, 2+, 3+, 4+	Pitting edema: mild, moderate, deep, very deep	Slide 15; Jarvis p. 518–519
6–23	Total Braden range; friction & shear max 3, others max 4	Braden handout
15–18 / 13–14 / 10–12 / ≤9	Braden: mild / moderate / high / severe	Slide 43; Braden handout
12 or less	Handout: total of 12 or less represents HIGH RISK	Braden handout
2.5 million / 60,000	Pressure injuries per year / deaths per year	Slide 40

Look-alike pairs

Pair	Difference
Vesicle vs bulla	Fluid-filled; vesicle ≤1 cm, bulla >1 cm [Jarvis p. 230]
Macule vs patch	Flat color change; <1 cm vs >1 cm [Jarvis p. 230]
Papule vs plaque	Solid raised; <1 cm vs papules coalescing >1 cm [Jarvis p. 230]
Papule vs nodule	Both solid elevated; nodule >1 cm and deeper in dermis [Jarvis p. 230]
Crust vs scale	Crust = dried exudate (honey/red-brown); scale = dry flakes of keratin [Jarvis p. 231]
Erosion vs ulcer	Erosion = shallow, epidermis only, no scar; ulcer = into dermis, leaves scar [Jarvis p. 232]
Excoriation vs erosion	Excoriation is self-inflicted abrasion from scratching [Jarvis p. 232]
Petechiae vs purpura vs ecchymosis	Petechiae 1–3 mm dots; purpura >3 mm confluent patch; ecchymosis >3 mm purple patch [Jarvis p. 237]
Contusion vs purpuric lesions	Bruise = blunt force trauma; petechiae/ecchymosis/purpura are not from blunt trauma [Jarvis p. 237]
Skin tag vs pedunculated lesion	Both on a stalk; skin tag is discrete, harmless [Jarvis p. 228]
Scar vs keloid	Keloid grows beyond the original wound edges [Jarvis p. 233]
Wheal vs urticaria	Urticaria = wheals coalescing [Jarvis p. 230]
Spider angioma vs cherry angioma	Spider = red center with radiating "legs," blanches; cherry = solid red dot 1–5 mm [Jarvis p. 211, 236]

Where to look in dark skin

Color change	Reliable sites
Pallor	Conjunctivae (near canthi), mucous membranes, nail beds, lips
Cyanosis	Conjunctivae, oral mucosa, nail beds
Jaundice	Sclera up to iris, hard/soft palate junction, palms
Erythema	Palpate for warmth, taut skin, deep hardening
Petechiae	Abdomen, buttocks, volar forearm; buccal mucosa, conjunctivae
Pressure injury stage 1	Skin darker but does not blanch; check over bony prominences

21. Likely test traps

- **ABCDEF** = Asymmetry, Border irregularity, Color variation, Diameter >6 mm, Elevation/Evolution, Funny looking ("ugly duckling") [Slide 12; Jarvis p. 207–208].
- **Braden: lower score = worse**; range 6–23 [Braden handout].
- **Friction & Shear maxes at 3** ("No apparent problem"); every other subscale maxes at 4 [Braden handout].
- **Braden bands:** mild 15–18, moderate 13–14, high 10–12, severe ≤9; "12 or less = high risk" [Slide 43; Braden handout].
- **Nail base angle 160° normal vs 180° early clubbing**; curved nail is 160° or less and is normal [Slide 8; Jarvis p. 214].
- **Capillary refill <2 s normal**; sluggish >3 s signals cardiovascular/respiratory failure or septic shock [Jarvis p. 215].
- **Papule <1 cm** (handout says ≤0.5 cm) vs **nodule >1 cm, deeper in dermis** [Slide 22–24; Lesion handout].
- **Vesicle ≤1 cm vs bulla >1 cm** — the 1 cm cutoff is the whole question [Slide 23].
- **Petechiae do not blanch** (extravasated blood); red macules from dilated vessels do [Jarvis p. 212, 237].
- **Purpura >3 mm**, confluent/patch of petechiae and ecchymoses [Jarvis p. 237].
- **Pitting grades 1+ to 4+** = mild, moderate, deep, very deep [Slide 15; Jarvis p. 518–519].
- **Turgor site: pinch a large fold on the anterior chest under the clavicle** — not the forearm [Slide 14; Jarvis p. 210].
- **Crust = dried exudate** (honey color in impetigo); **scale = flakes of keratin** [Slide 26; Jarvis p. 231].
- **Erosion heals without a scar; ulcer leaves a scar** [Slide 27–28; Jarvis p. 232].
- **Keloid spreads past the original wound**; atrophic scar is depressed/thinned (striae) [Slide 31; Jarvis p. 233].
- **Zosteriform = linear along a unilateral dermatome** (shingles) [Slide 21; Jarvis p. 229].
- **Confluent = lesions run together** (hives) [Slide 18; Jarvis p. 228].

- **Diaphoresis** = profuse sweating (fever, heart attack, anxiety, pain) [Slide 13; Jarvis p. 210].
- **Stage 1 pressure injury is non-blanchable erythema**; in dark skin the area is darker but does not blanch [Jarvis p. 234].
- **Mucosal pressure injuries cannot be staged**; skin PIs are staged [Jarvis p. 234].

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22. Module 3 vocabulary (31 terms)

> Study-only answers. If this list gets turned in, rewrite each definition in your own words (NRSG 233 AI policy).

#	Term	Meaning	Source
1	**alopecia**	Hair loss, patchy or total (e.g., chemotherapy, fungal infection, autoimmune disease)	Jarvis p. 213
2	**bulla**	Raised, fluid-filled blister larger than 1 cm (burn blister, pemphigus)	Slide 23; Jarvis p. 230
3	**capillary refill**	Return of color after pressing and releasing the nail bed; normal 1–2 seconds (assess <2 s); slow refill suggests poor peripheral circulation	Slide 8; Jarvis p. 215
4	**cherry angioma**	Small (1–5 mm), smooth, bright red raised dot on the trunk; common, harmless, increases after about age 30	Slide 32; Jarvis p. 211
5	**clubbing**	Nail base angle reaches 180° or more with a spongy base and swollen fingertip; from chronic heart or lung disease; normal angle about 160°	Slide 8; Jarvis p. 214
6	**comedo**	Plugged hair follicle (pore) filled with oil and keratin; open = blackhead, closed = whitehead; first lesion of acne	Jarvis p. 228 (acne)
7	**confluent**	Lesions that run together and merge (e.g., hives)	Slide 18; Jarvis p. 228
8	**cyanosis**	Bluish skin/mucous color from too little oxygen in the blood; in dark skin check lips, tongue, conjunctiva, palms, soles	Slide 11; Jarvis p. 208–209
9	**cyst**	Encapsulated raised lump of fluid or semisolid material in the dermis or subcutaneous layer (e.g., sebaceous cyst)	Slide 25; Jarvis p. 231
10	**diaphoresis**	Heavy, profuse sweating (fever, exercise, anxiety, shock, heart attack)	Jarvis p. 210

#	Term	Meaning	Source
11	**erythema**	Redness from increased blood in dilated superficial capillaries (inflammation, fever, sunburn, blushing); in dark skin feel for warmth, tautness, swelling	Slide 11; Jarvis p. 208
12	**hematoma**	Collection of blood trapped under the skin from a broken vessel, forming a raised, bruise-like lump	Jarvis p. 208, 237
13	**homogeneous**	Even and the same all over (uniform skin color; a lesion with even color throughout)	Slide 10; Jarvis p. 207
14	**jaundice**	Yellow skin, sclera, and mucous membranes from high bilirubin; check sclera first, especially in dark skin	Slide 11; Jarvis p. 209
15	**keloid**	Overgrown raised scar that spreads past the original wound edges; smooth, rubbery; more common with darker skin	Slide 31; Jarvis p. 233
16	**macule**	Flat spot, color change only, under 1 cm, cannot be felt (freckle, flat mole); flat and larger than 1 cm = patch	Slide 22; Jarvis p. 230
17	**nevus (mole)**	Cluster of melanocytes, tan to brown, flat or raised; screen with ABCDEF for melanoma	Slide 9; Jarvis p. 207
18	**pallor**	Paleness from less blood flow or low hemoglobin (anemia, shock, cold); ashen gray or yellowish brown in dark skin; check conjunctiva, lips, nail beds	Slide 11; Jarvis p. 208
19	**petechiae**	Tiny flat red-purple dots (1–3 mm) from bleeding under the skin; do not blanch; may mean a bleeding/clotting problem	Slide 34; Jarvis p. 237

#	Term	Meaning	Source
20	**primary lesion**	Lesion arising directly from a disease process on previously normal skin (macule, papule, vesicle, wheal, nodule)	Slide 22; Jarvis p. 211, 230
21	**purpura**	Flat red-purple patch larger than petechiae (>3 mm) from bleeding under the skin; does not blanch; low platelets or vessel problems	Slide 34; Jarvis p. 237
22	**secondary lesion**	Change that develops from a primary lesion over time or from scratching, healing, or infection (crust, scale, scar, ulcer, fissure, excoriation, lichenification)	Slide 26; Jarvis p. 211, 231
23	**skin tags**	Soft small flaps of skin on a stalk, common on neck, armpits, eyelids; harmless	Jarvis p. 228
24	**skin tenting**	Pinched skin stays up in a ridge instead of snapping back — poor turgor, usually dehydration	Jarvis p. 210
25	**spider angioma**	Fiery red star-shaped marking with solid center and radiating "legs" that blanch with pressure; face, neck, chest; liver disease, pregnancy, B vitamin deficiency, sometimes normal	Slide 32; Jarvis p. 236
26	**striae**	Silvery or pink stretch marks from rapid stretching (pregnancy, weight gain, growth); purple-blue ones can point to Cushing syndrome	Jarvis p. 233 (atrophic scar)
27	**turgor**	Skin elasticity, tested by pinching a fold under the clavicle; normal snaps right back, slow return = dehydration	Slide 14; Jarvis p. 210
28	**vesicle**	Raised fluid-filled blister under 1 cm (chickenpox, herpes, shingles)	Slide 23; Jarvis p. 230

#	Term	Meaning	Source
29	**vitiligo**	Patchy loss of pigment (white areas) from destruction of melanocytes, thought autoimmune; more noticeable on dark skin	Slide 9; Jarvis p. 207, 226
30	**wheal**	Raised, red, itchy, irregular area of transient swelling (hives, mosquito bite, allergic reaction)	Slide 23; Jarvis p. 230
31	**zosteriform**	Lesions in a band along a nerve path (dermatome), usually one-sided, like shingles	Slide 21; Jarvis p. 229

Appendix A

Skin Lesion Reference Guide, her handout (shared with permission from Relias/Wound Care Education Institute). Unchanged.

Skin Lesion Reference Guide



Bulla
Circumscribed collection of free fluid, >1 cm



Macule
Circular flat discoloration, <1 cm brown, blue, red or hypopigmented



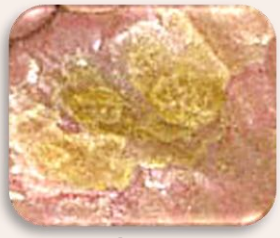
Nodule
Circular, elevated, solid lesion, >1cm



Patch
Circumscribed flat discoloration, >1cm



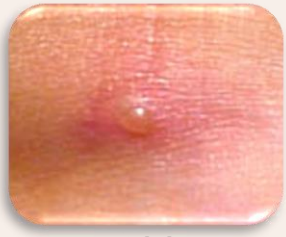
Papule
Superficial solid elevated, ≤0.5 cm, color varies



Plaque
Superficial elevated solid flat topped lesion, >1 cm



Pustule
Vesicle containing pus (inflammatory cells)



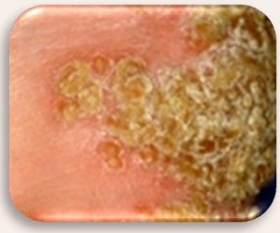
Vesicle
Circular collection of free fluid, ≤1 cm



Wheal
Edematous, transitory plaque, may last few hours



Scale
Epidermal thickening; consists of flakes or plates of compacted desquamated layers of stratum corneum



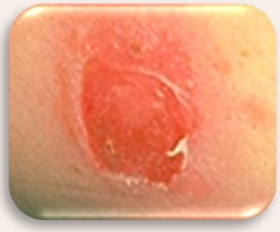
Crust
Dried serum or exudate on skin



Fissure
Crack or split



Excoriation
Linear erosion



Erosion
Loss of epidermis (superficial); Part or all of the epidermis has been lost



Lichenification
Thickening of the epidermis seen with exaggeration of normal skin lines



Scar
Thickening; permanent fibrotic changes that occur on the skin following damage to the dermis

#DRM806 ©2006 Wound Care Education Institute – www.wcei.net

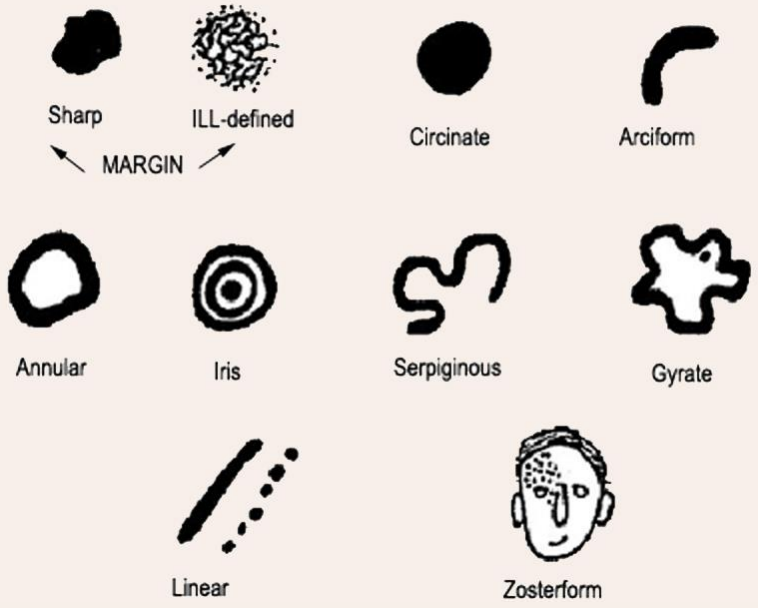
Inspection/Documentation of Lesions:

- Color
- Number of lesions (the actual number, or estimation)
- Location and distribution of lesions – be specific! (e.g., scalp, face and trunk are areas of high sebaceous gland concentration; on extremities, it makes a difference whether lesions are on extensor or flexor surfaces
- Grouping or configuration of lesions (e.g., linear, annular)
- Size of lesions in centimeters
- Shape and borders (e.g., round, linear, oval, and geographic are shapes; borders can be well-defined, ill-defined, circumscribed, or irregular)
- Surface (e.g., flat or raised)
- Tenderness (or non-tender)
- Mobility (mobile – moves with the skin; non-mobile or fixed – skin moves over it)
- Special characteristics (e.g., pediculated (on a stalk), exudative. etc.)

Configuration

Refers to the shape or outline of lesions

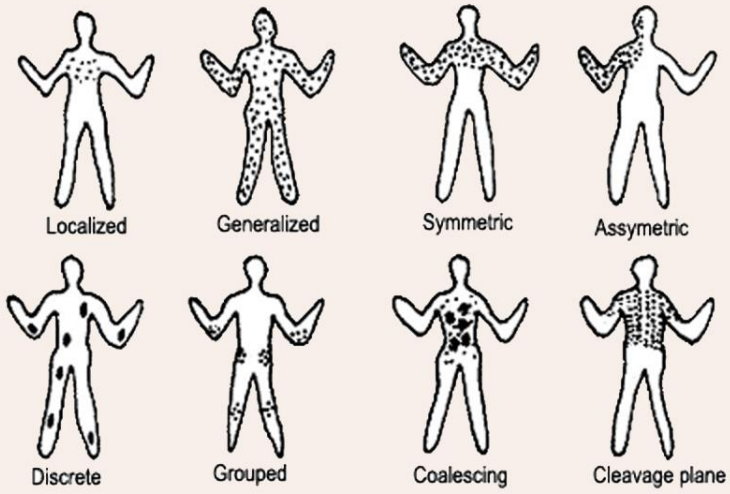
- Annular - round or circular with central clearing
- Circinate - round, circular > arciform: partial circle
- Iris or target -also known as target lesions and are a series of concentric rings. These have a dark or blistered center.
- Gyrate - connecting arcs
- Linear - straight
- Serpiginous – meandering; wander as though following the track of a snake.
- Margination – sharp, ill-defined?
- Satellite Lesions - commonly used to describe a portion of the rash of cutaneous candidiasis in which a beefy red plaque may be found surrounded by numerous, smaller red macules located adjacent to the body of the main lesions
- Zosteriform - dermatomal



Distribution

The clinical arrangement of skin lesions

- Localized - grouped into specific areas
- Generalized - dispersed all over
- Symmetric - no pattern
- Asymmetric - pattern lacking randomness
- Discrete - separate
- Grouped - clustered
- Confluent (coalescing) - smaller into larger
- Cleavage plane - arranged along lines of skin tension



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Appendix B

Braden Scale for Predicting Pressure Sore Risk, her handout. Unchanged.

BRADEN SCALE – For Predicting Pressure Sore Risk

SEVERE RISK: Total score ≤ 9 **HIGH RISK**: Total score 10-12
MODERATE RISK: Total score 13-14 **MILD RISK**: Total score 15-18

RISK FACTOR	SCORE/DESCRIPTION				DATE OF ASSESS ➡			
					1	2	3	4
SENSORY PERCEPTION Ability to respond meaningfully to pressure-related discomfort	**1. COMPLETELY LIMITED** – Unresponsive (does not moan, flinch, or grasp) to painful stimuli, due to diminished level of consciousness or sedation, **OR** limited ability to feel pain over most of body surface.	**2. VERY LIMITED** – Responds only to painful stimuli. Cannot communicate discomfort except by moaning or restlessness, **OR** has a sensory impairment which limits the ability to feel pain or discomfort over ½ of body.	**3. SLIGHTLY LIMITED** – Responds to verbal commands but cannot always communicate discomfort or need to be turned, **OR** has some sensory impairment which limits ability to feel pain or discomfort in 1 or 2 extremities.	4. **NO IMPAIRMENT** – Responds to verbal commands. Has no sensory deficit which would limit ability to feel or voice pain or discomfort.				
MOISTURE Degree to which skin is exposed to moisture	1. **CONSTANTLY MOIST**– Skin is kept moist almost constantly by perspiration, urine, etc. Dampness is detected every time patient is moved or turned.	2. **OFTEN MOIST** – Skin is often but not always moist. Linen must be changed at least once a shift.	3. **OCCASIONALLY MOIST** – Skin is occasionally moist, requiring an extra linen change approximately once a day.	4. **RARELY MOIST** – Skin is usually dry; linen only requires changing at routine intervals.				
ACTIVITY Degree of physical activity	1. **BEDFAST** – Confined to bed.	2. **CHAIRFAST** – Ability to walk severely limited or nonexistent. Cannot bear own weight and/or must be assisted into chair or wheelchair.	3. **WALKS OCCASIONALLY** – Walks occasionally during day, but for very short distances, with or without assistance. Spends majority of each shift in bed or chair.	4. **WALKS FREQUENTLY**– Walks outside the room at least twice a day and inside room at least once every 2 hours during waking hours.				
MOBILITY Ability to change and control body position	1. **COMPLETELY IMMOBILE** – Does not make even slight changes in body or extremity position without assistance.	2. **VERY LIMITED** – Makes occasional slight changes in body or extremity position but unable to make frequent or significant changes independently.	3. **SLIGHTLY LIMITED** – Makes frequent though slight changes in body or extremity position independently.	4. **NO LIMITATIONS** – Makes major and frequent changes in position without assistance.				
NUTRITION Usual food intake pattern [1]NPO: Nothing by mouth. [2]IV: Intravenously. [3]TPN: Total parenteral nutrition.	**1. VERY POOR** – Never eats a complete meal. Rarely eats more than 1/3 of any food offered. Eats 2 servings or less of protein (meat or dairy products) per day. Takes fluids poorly. Does not take a liquid dietary supplement, **OR** is NPO[1] and/or maintained on clear liquids or IV[2] for more than 5 days.	**2. PROBABLY INADEQUATE** – Rarely eats a complete meal and generally eats only about ½ of any food offered. Protein intake includes only 3 servings of meat or dairy products per day. Occasionally will take a dietary supplement **OR** receives less than optimum amount of liquid diet or tube feeding.	**3. ADEQUATE** – Eats over half of most meals. Eats a total of 4 servings of protein (meat, dairy products) each day. Occasionally refuses a meal, but will usually take a supplement if offered, **OR** is on a tube feeding or TPN[3] regimen, which probably meets most of nutritional needs.	4. **EXCELLENT** – Eats most of every meal. Never refuses a meal. Usually eats a total of 4 or more servings of meat and dairy products. Occasionally eats between meals. Does not require supplementation.				
FRICTION AND SHEAR	1. **PROBLEM**- Requires moderate to maximum assistance in moving. Complete lifting without sliding against sheets is impossible. Frequently slides down in bed or chair, requiring frequent repositioning with maximum assistance. Spasticity, contractures, or agitation leads to almost constant friction.	2. **POTENTIAL PROBLEM**– Moves feebly or requires minimum assistance. During a move, skin probably slides to some extent against sheets, chair, restraints, or other devices. Maintains relatively good position in chair or bed most of the time but occasionally slides down.	3. **NO APPARENT PROBLEM** – Moves in bed and in chair independently and has sufficient muscle strength to lift up completely during move. Maintains good position in bed or chair at all times.					
TOTAL SCORE	**Total score of 12 or less represents HIGH RISK**							

ASSESS	DATE	EVALUATOR SIGNATURE/TITLE	ASSESS.	DATE	EVALUATOR SIGNATURE/TITLE
1	/ /		3	/ /	
2	/ /		4	/ /	

NAME-Last	First	Middle	Attending Physician	Record No.	Room/Bed

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BRADEN SCALE

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